

Hypertension – management in pregnancy guideline (GL952)

Approval

Approval Group	Job Title, Chair of Committee	Date
Maternity & Children's Services Clinical Governance Committee	Chair, Maternity Clinical Governance Committee	13 th December 2024 & 13/3/25

Change History

Version	Date	Author, job title	Reason
6.0	May 2024	S Wong, Consultant O&G S Williams, Consultant Anaesthetist	Biennial review – changes to pg 31 regarding Red Box and pg 34 changing criteria for checking MgSO4 levels
6.1	March 2025	S Wong / S Williams	Live change to update EMA080 flowchart
6.2	Oct 2025	S Wong, Consultant Obstetrician	Live change to add specification for women first presented with PET to be reviewed by registrar (pg 16). Also changed tinzaparin to LMWH throughout and updated the duration of magnesium sulphate (pg 34)
6.3	March 2026	S Wong, Consultant Obstetrician	Live change to pg 5 to add criteria to consider other renal pathology in women with proteinuria and is normotensive in response to PM case

To be read in conjunction with the following guidelines:

- [Fetal monitoring guideline \(GL964\)](#)
- [Magnesium Sulphate for neonatal neuroprotection \(GL868\)](#)
- [Pre-term, pre-labour rupture of the membranes \(PPRoM\) guideline \(GL895\)](#)
- [Steroid Prophylaxis Against RDS Guideline \(GL920\)](#)

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CONTENTS

Quick look guides

First presentation and outpatient management care pathway (ANC and DAU) for Chronic Hypertension.....	6
In-patient care pathway: Severe Chronic Hypertension	7
ANTENATAL In-patient care pathway: Chronic Hypertension	8
POSTNATAL In-patient care pathway: Chronic Hypertension	8
INTRAPARTUM care pathway: Chronic Hypertension	9
First presentation and outpatient management care pathway: Gestational Hypertension	11
In-patient care pathway: Severe Gestational Hypertension	12
ANTENATAL In-patient care pathway: Gestational Hypertension	13
POSTNATAL In-patient care pathway: Gestational Hypertension	13
INTRAPARTUM care pathway: Gestational Hypertension.....	14
First presentation and outpatient management care pathway: Pre-Eclampsia	15
In-patient care pathway: Pre-Eclampsia (moderate- severe)	16
ANTENATAL In-patient care pathway: Pre-Eclampsia	17
POSTNATAL In-patient care pathway: Pre-Eclampsia	18
INTRAPARTUM WARD care pathway: Pre-eclampsia	19
LABOUR WARD care pathway: Severe Hypertension, Severe Pre-eclampsia and Eclampsia	20
POSTNATAL care pathway on labour ward: Severe hypertension, Severe pre-eclampsia and Eclampsia	21
1.0 Overview	22
1.1 Definitions:-	22
1.2 Taking the blood pressure:-.....	23
1.3 Urinalysis:-.....	23
1.4 Blood tests:-	23
1.5 Ultrasound scan:-	23
1.6 Management pathways:	24
1.7 Reducing the risk of hypertensive disorders in pregnancy: -	24
1.8 Treatment of Hypertension: -.....	25
2.0 Acute Management of Hypertension	26
3.0 Management of antenatal inpatients with hypertension.....	27
3.1 Antenatal inpatient Care Pathway: Severe Chronic Hypertension (page 8)	27
3.2 Antenatal in-patient care pathway: Gestational Hypertension (page 13).....	28
3.3 Antenatal care after discharge:.....	28
3.4 Antenatal Inpatient care pathway: Pre-eclampsia (page 17)	28
4.0 Intrapartum care	29
4.1 Mild or moderate hypertension (BP 140/90-159/109 mmHg)	29
4.2 Immediate postnatal care on the labour ward.....	30
5.0 Management of Severe Hypertension and Severe Pre eclampsia / eclampsia on labour ward (page 21).....	30

Author:	S Wong, S Williams	Date:	October 2025
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.2
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

5.1 Overview:	30
5.2 Definitions:	30
5.3 Anti-hypertensive therapy:	31
5.4 Anticonvulsants	32
5.4.1 Immediate management of an eclamptic fit and magnesium sulphate infusion	33
5.4.2 Further seizures while on magnesium sulphate infusion	33
5.4.3 Monitoring during MgSO ₄ therapy	34
5.5 Blood pressure:	35
5.6 O ₂ saturation levels:	35
5.7 Fluid Balance:	35
5.8 Urine output:	36
5.9 Timing of delivery:	36
5.10 Analgesia	37
5.11 Coagulation control	37
5.12 Anaesthesia for delivery	37
5.13 Post-partum	40
6.0 POSTNATAL Labour Ward care pathway: Severe Hypertension, severe pre-eclampsia and Eclampsia (page 20/21)	40
6.1 Immediate postnatal care of women who have received MgSO ₄	40
6.2 Immediate postnatal care on the labour ward of women with severe hypertension and/or eclampsia	40
6.3 Postnatal care pathway on labour ward:	41
7.0 In-patient postnatal care	41
7.1 Postnatal ward management of hypertensive women	41
7.2 Postnatal blood pressure management:	42
7.3 Maintenance of blood pressure:	42
8.0 Postnatal care following discharge from hospital	44
8.1 Women with Chronic Hypertension	44
8.2 Women with Gestational Hypertension	44
8.3 Women with Pre-Eclampsia	45
9.0 References	46
10.0 Monitoring Appendices and tables	46
Appendix 1:.....Indication for early delivery in a woman with pre-eclampsia who require in-patient management	47
Appendix 2: Discharge to GP letter	48
Table 1: Antenatal risk reduction	49
Table 2: Classification of hypertensive disorders and summary of antenatal antihypertensive options	50
Table 3: Management of antenatal hypertension	51
Table 4: Diagnosis and management of severe hypertension: Antihypertensive treatment options	52

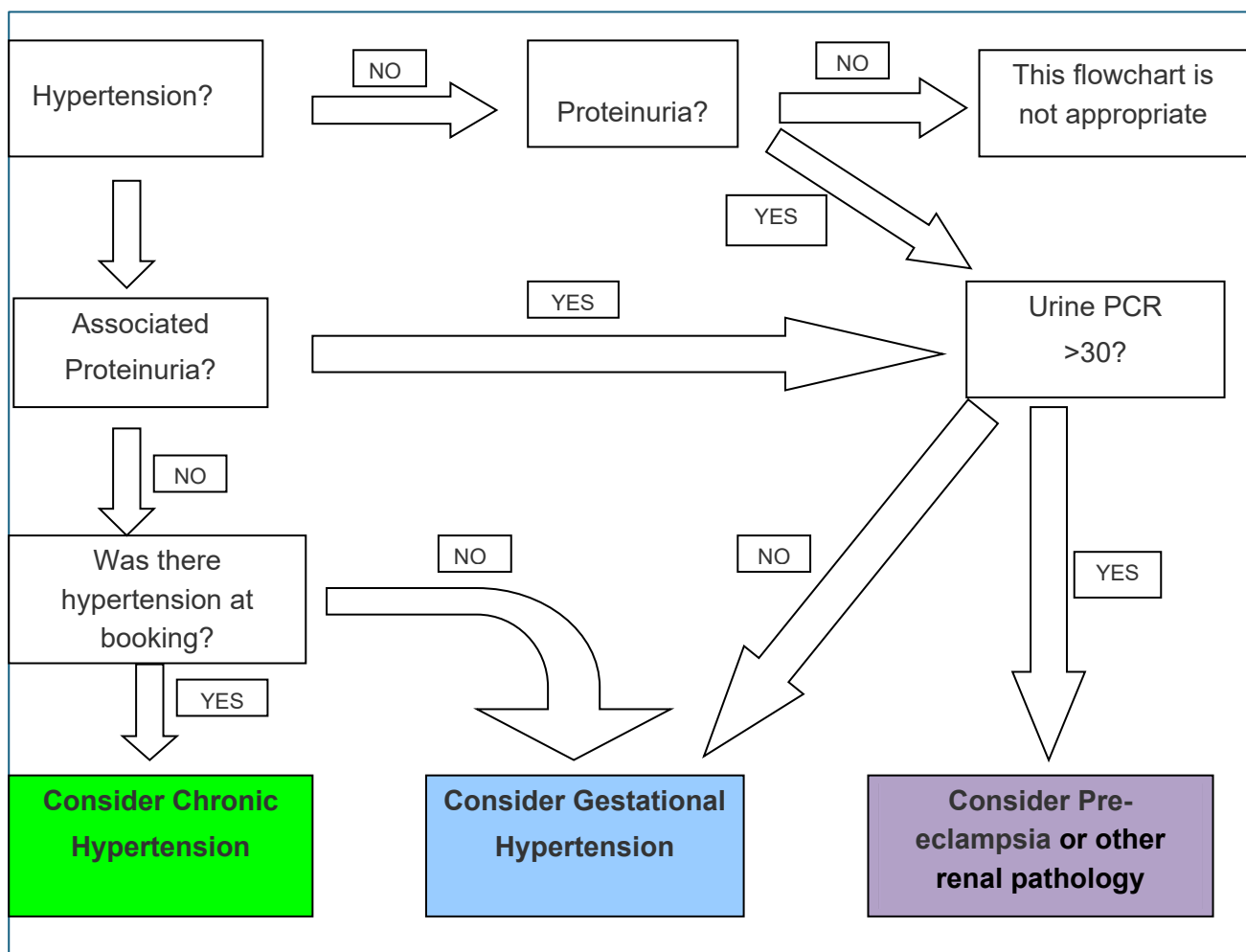
Author:	S Wong, S Williams	Date:	March 2026
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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 5: Management of severe hypertension: assessment, diagnosis and fluid balance..	53
Table 6: Management of severe hypertension: Eclampsia:	54
Table 7: Fetal assessment and delivery planning	55
Table 8: Summary of postnatal hypertension management.....	56
Table 9: Antihypertensive therapy and breastfeeding	57
Table 10: Recurrence risks of hypertension and long-term health risks	58

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
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First presentation and outpatient antenatal care

Referral Sign/Symptom: -



If the midwife/registrar could not agree on the most suitable management care pathway they must discussed this with a consultant.

At each presentation, the woman must be assessed using the above flow chart to ensure that the correct management care pathway is followed. Remember women with chronic hypertension or gestational hypertension can develop pre-eclampsia (if that is the case change the management care pathway to PET).

Management must follow the documented pathway unless a consultant decides that the usual management pathway is not appropriate (see overview above). Please consider other renal pathology in women with proteinuria and is normotensive

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First presentation and outpatient management care pathway (ANC and DAU) for Chronic Hypertension

Degree of hypertension	Care			Escalation to medical staff
	Mild hypertension BP 140-90-149/99 mmHg	Moderate hypertension BP 150/100-159-109 mmHg	Severe hypertension BP $\geq$ 160/110 mmHg	
Admit to Hospital	No	No	Yes (until BP is $<$ 140/90)	
Blood pressure measurement	Most women with chronic hypertension will already be under the care of a consultant and have a management care pathway in place. Aim for BP $<$ 140/90mmHg			If BP $\geq$ 140/90mmHg a registrar / Consultant review is required and a change in medication needs to be considered
Treatment	Continue antenatal antihypertensive treatment throughout the pregnancy and review long-term antihypertensive treatment 2 weeks after the birth. Offer women with chronic hypertension a medical review at the postnatal review (6–8 weeks after the birth) with pre-pregnancy counselling			
Urinalysis	Check at each visit. When a result of 1+ protein or more is obtained, proteinuria must be quantified by urinary protein: creatinine ratio (PCR)		If the PCR is $>$ 30mg/mmol (and the woman does not have renal disease) this indicates that she has developed pre-eclampsia and must now be managed on the PET care pathway .	
Blood tests	A baseline PET screen should be sent at first diagnosis. This should not be repeated unless clinically indicated.			
Fetal Monitoring	Fetal echocardiogram at 22-24 weeks if on treatment Ultrasound scan for fetal growth, liquor volume and umbilical artery Doppler should be performed at 28-weeks and at 32-weeks and 36 weeks' gestations, =. Cardiotocography (CTG) only if clinically indicated		If the CTG is not normal it must be promptly reviewed by obstetric registrar and may need to be discussed with an obstetric consultant.	
Timing of birth	If BP $<$ 160/110mmHg with or without anti-hypertensive treatment: - do not offer delivery before 37 weeks - After 37 weeks timing of delivery should be decided between the woman and the senior obstetrician, discussion of maternal and fetal indications for birth should be documented on EPR. If BP $\geq$ 160/110mmHg despite optimum antihypertensive treatment (refractory), offer birth after course of corticosteroids and magnesium sulphate for neuroprotection if required in accordance with guideline GL868			

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In-patient care pathway: Severe Chronic Hypertension

Care		Escalation to medical staff
Complete a VTE risk assessment	If LMWH is indicated, prescribe at 2200hrs daily.	
Blood pressure measurement	Take and record blood pressure 4 hourly	If BP $\geq$ 150/100mmHg: inform SHO, who should review the woman within 1 hour.
Urinalysis	Daily urinalysis - If proteinuria of 1+ or more send a urine sample to the biochemistry lab for an urgent protein: creatinine ration (PCR). - If the PCR is $>30\text{mg}/\text{mmol}$ (and the woman does not have renal disease) this indicates that she has developed pre-eclampsia and must now be managed on the PET care pathway.	The registrar must be informed of this change at the next ward round (earlier if clinical concerns).
Blood tests	PET screen on day of admission. If the woman remains in hospital, repeat PET screen weekly.	Must be reviewed on EPR
Fetal Monitoring	CTG on admission if normal do not repeat unless clinically indicated Only repeat CTG if the woman reports: - Change in fetal movements - Vaginal bleeding - Abdominal pain - Deterioration in maternal condition If there is concern about fetal growth the frequency for CTG/ scan monitoring will be decided by the obstetric team	If the CTG is not normal it must be promptly reviewed by an obstetric registrar who will discuss with an obstetric consultant
	Ultrasound scan for fetal growth, liquor volume and umbilical artery Doppler should be performed at 28-30 weeks and at 32-34 weeks' gestations. Extra scans are not necessary on inpatients unless there are specific clinical concerns. Ultrasound scan reports must be reviewed by obstetric registrar or consultant within 24 hours .	Borderline or abnormal results must be discussed with a consultant.
Preparation for early delivery	Administer corticosteroids in accordance to the Steroid Prophylaxis Against RDS (GL920) .	

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ANTENATAL In-patient care pathway: Chronic Hypertension

QUICK REFERENCE GUIDE

- Complete VTE assessment and give LMWH at 22.00hrs if indicated
- 4 hourly blood pressure measurement
- Daily urinalysis
- Weekly PET screen
- CTG on admission
- Only perform repeat CTG if the woman reports reduced fetal movements, vaginal bleeding, abdominal pain, deterioration in maternal condition.
- Ultrasound scan – fetal growth, liquor volume and umbilical artery Doppler should be performed at 28-30 weeks and at 32-34 weeks' gestations. If results are normal, do not repeat at more than 34 weeks, unless otherwise clinically indicated.

POSTNATAL In-patient care pathway: Chronic Hypertension

QUICK REFERENCE GUIDE

- Complete postnatal VTE assessment and give LMWH if indicated
- 4 hourly blood pressure measurement first day, then once a day while in patient or as clinically indicated if treatment changed then at least once between day 3-5 after discharge. Ask about symptoms at each BP check
- Aim to maintain BP $\leq$ 140/90 mmHg
- No extra blood tests unless clinical concern
- If the woman on methyldopa during pregnancy, stop within 2 days of birth and restart the antihypertensive treatment she was taking before the pregnancy.
- Postnatal stay – must be > 24hrs since last increase in medication.
- Before discharge, generate a 'postnatal blood pressure management plan.'
- Review long term antihypertensive treatment 2 weeks after birth
- Offer women with chronic hypertension a medical review at the postnatal review (6–8 weeks after the birth)

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INTRAPARTUM care pathway: Chronic Hypertension

Mild or moderate hypertension (BP 140/90-159/109mmHg)

Care		Escalation to medical staff
Blood pressure measurement	- Hourly BP and record on partogram - If BP $\geq$ 160/110 Hg inform obstetric registrar - If BP stable do not routinely limit duration of second stage	Review woman and transfer care management to Severe hypertension pathway If BP does not respond to initial treatment operative birth should be considered unless delivery is imminent.
Medication	- Continue antenatal antihypertensive treatment if any - Use 10iu Oxytocin IM for active management of third stage	
Urine	- If urinalysis shows an unexpected 1+ or more protein: - If practical arrange urgent urine PCR - If the PCR is $>30\text{mg}/\text{mmol}$ (and the woman does not have renal disease) this indicates that she has developed pre-eclampsia and must now be managed on the PET care pathway. - Follow bladder care guidelines	Woman to be examined by the registrar if develops significant proteinuria.
Blood tests	If urinalysis shows an unexpected 1+ or more protein: PET screen (FBC, U&E, LFT)	Review blood results
Fetal Monitoring	- CTG on admission for a minimum of 30 minutes if normal then use intermittent auscultation in labour - In established labour, Intermittent CTG/auscultation If FH not normal transfer to continuous CTG.	Follow fetal monitoring guideline GL964
VTE risk assessment	- Complete a VTE risk assessment. - If epidural is considered, do not site until 12 hours if LMWH has been administered.	
Immediate postnatal care	- Blood pressure to be taken within an hour of delivery and document on MOWS chart. - Aim to maintain BP $<140/90\text{mmHg}$ - Transfer to Iffley ward when clinically stable - A clear plan of care must be documented in the EPR record prior to transfer to Iffley ward	- If methyldopa was used during pregnancy, stop it and change it to another antihypertensive treatment (pre-pregnancy medication). - The registrar (or SHO after consultation with the registrar) must document an alternative antihypertensive regime in the woman's notes. - If methyldopa was not used during the antenatal period, continue antenatal hypertensive treatment

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Chronic kidney disease

Assess renal function at least every 24 hours during the intrapartum period in all women with chronic kidney disease because prolonged labour may lead to dehydration and acute kidney injury.

Consider planned birth by 40+0 weeks of pregnancy for women with:

- chronic kidney disease stage 1 and nephrotic-range proteinuria (urine protein: creatinine ratio greater than 300 mg/mmol) or
- chronic kidney disease stage 2 to 4 with stable renal function.

For all women with kidney disease, including those with a kidney transplant, base decisions on mode of birth on the woman's preference and obstetric indications.

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First presentation and outpatient management care pathway: Gestational Hypertension

Care				Escalation to medical staff
Degree of hypertension	Mild hypertension BP 140-90-149/99mmHg	Moderate hypertension BP 150/100-159/109 mmHg	Severe hypertension BP >160/110 mmHg	
Admit to Hospital	No	No	Yes (until BP is 159/109 or lower)	
Blood pressure measurement	After 32 weeks: Weekly with CMW Prior to 32 weeks: Twice weekly with CMW	Twice weekly with CMW	Admit to hospital, At least four times a day Once controlled and discharged check twice weekly	If seen in DAU on 3 occasions, referral to Consultant ANC for further assessment
Urinalysis	Check at each visit. When a result of 1+ protein or more is obtained, proteinuria must be quantified by urinary protein: creatinine ratio If the PCR is >30mg/mmol (and the woman does not have renal disease) this indicates that she has developed pre-eclampsia and must now be managed on the PET care pathway . If there is uncertainty about the diagnosis of pre-eclampsia, consider re-testing on a new sample alongside clinical review.			
Blood tests	Routine antenatal blood tests only	Baseline PET screening (FBC, U&E, LFT) Do not carry out further blood tests if no proteinuria at subsequent visits	Send PET screen (FBC, U&E, LFT) Once BP controlled repeat PET screen weekly	
Medication	Offer labetalol if BP remains above 140/90mmHg. Once on medication aim for BP of 135/85 or less.			
Fetal Monitoring	If diagnosis confirmed before 34 weeks: Ultrasound scan for fetal growth If results normal do not repeat after 34 weeks. CTG should not be performed unless the woman reports decreased fetal movements / vaginal bleeding / abdominal pain / deterioration in maternal condition		At diagnosis: Ultrasound scan for fetal growth. Do not repeat more than every 2 weeks. CTG at first presentation	
Timing of birth	If BP <160/110mmHg with or without anti-hypertensive treatment: - do not offer delivery before 37 weeks - After 37 weeks timing of delivery should be decided between the woman and the senior obstetrician, discussion of maternal and fetal indications for birth should be documented on EPR record. If BP ≥160/110mmHg despite optimum antihypertensive treatment (refractory), offer birth after course of corticosteroids and magnesium sulphate for neuroprotection if required in accordance with guideline GL868			

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In-patient care pathway: Severe Gestational Hypertension

Care		Escalation to medical staff
Complete a VTE risk assessment	If LMWH is indicated, prescribe at 22.00hrs daily.	
Blood pressure measurement	Take and record blood pressure 4 hourly daily.	If BP $\geq$ 140/90mmHg: Inform SHO, who should review the woman within 1 hour.
Urinalysis	Daily urine dipstick - If proteinuria of 1+ or more send a urine sample to the biochemistry lab for an urgent protein: creatinine ration (PCR). - If the PCR is >30mg/mmol (and the woman does not have renal disease) this indicates that she has developed pre-eclampsia and must now be managed on the PET care pathway. If there is uncertainty about the diagnosis of pre-eclampsia, consider re-testing on a new sample alongside clinical review.	The registrar must be informed of this change at the next ward round (earlier if clinical concerns).
Blood tests	PET screen on day of admission. If the woman remains in hospital, repeat PET screen weekly.	Must be reviewed on EPR
Fetal Monitoring	CTG – on day of admission. Do not repeat if a normal CTG has already been recorded that day. Repeat CTG weekly unless the woman reports: - Change in fetal movements - Vaginal bleeding - Abdominal pain - Deterioration in maternal condition If there is concern about fetal growth the regime for CTG/scan monitoring will be decided by the obstetric team If the CTG is repeated at <1 week then indication must be recorded on the CTG and in the woman's notes.	If the CTG is not normal it must be promptly reviewed by an obstetric registrar who will discuss with an obstetric consultant
	Ultrasound scan for fetal growth, liquor volume and umbilical artery Doppler should be arranged within 2 days of admission. Do not repeat more frequently than every 2 weeks if normal. Ultrasound scan reports must be reviewed by obstetric registrar or consultant within 24 hours .	Borderline or abnormal results must be discussed with a consultant.
Preparation for early delivery	Administer corticosteroids and magnesium sulphate for neuroprotection if required in accordance with guideline GL868	

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ANTENATAL In-patient care pathway: Gestational Hypertension

QUICK REFERENCE GUIDE

- Complete VTE assessment and give LMWH at 22.00hrs if indicated
- 4 hourly blood pressure measurement
- Daily urinalysis
- Weekly PET screen
- CTG on admission
- Only repeat if the CTG is abnormal or there are changes in the woman condition e.g. she reports reduced fetal movements, vaginal bleeding, abdominal pain, deterioration in maternal condition
- Ultrasound scan- fetal growth, liquor volume and umbilical artery Doppler should be arranged within 2 days of admission. Do not repeat more frequently than every 2 weeks if normal.

POSTNATAL In-patient care pathway: Gestational Hypertension

QUICK REFERENCE GUIDE

- Complete postnatal VTE assessment and give LMWH if indicated
- 4 hourly blood pressure measurement first day then once a day while in patient or as clinically indicated if treatment changed, then at least once between day 3-5 after discharge. Ask about symptoms at each BP check
- Aim to maintain BP $\leq 149/99$ mmHg
- If BP $<130/80$ mmHg for 24hrs, reduce antihypertensive medication
- Start antihypertensive if BP $> 149/99$ if not already on treatment
- If the woman on methyldopa during pregnancy, stop within 2 days of birth and change to Labetalol or Nifedipine or ACE inhibitors
- No extra blood tests unless clinical concern
- Postnatal stay – must be > 24 hrs since last increase in medication
- Before discharge generate a 'postnatal blood pressure management plan'
- If still on antihypertensive treatment 2 weeks after discharge will need medical review
- Will need medical review 6-8 weeks after the birth and referral to hypertension specialist if still needing antihypertensive treatment

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INTRAPARTUM care pathway: Gestational Hypertension

Mild or moderate hypertension (BP 140/90-159/109mmHg)

Care		Escalation to medical staff
Blood pressure measurement	- Hourly BP, document on partogram - If BP $\geq$ 160/110 Hg inform obstetric registrar - If BP stable do not routinely limit duration of second stage	If BP $\geq$ 160/110 Hg review woman and transfer care management to Severe hypertension pathway If BP does not respond to initial treatment operative birth is recommended.
Medication	- Continue antenatal antihypertensive treatment if any - Use 10iu Oxytocin IM for active management of third stage	If BP $\geq$ 150/100 mmHg and no previous antenatal treatment was prescribed then antihypertensive treatment should be commenced
Urine	- If urinalysis shows an unexpected 1+ or more protein: - This indicates that she has developed pre-eclampsia and must now be managed on the PET care pathway. If there is uncertainty about the diagnosis of pre-eclampsia, consider re-testing on a new sample alongside clinical review.	Woman to be examined by the registrar if develops significant proteinuria.
Blood tests	Take blood for PET screen and a Group and Save on admission to labour ward unless these have been taken within last 24 hours.	Review blood results.
Fetal Monitoring	- CTG on admission for a minimum of 30 minutes if normal then intermittent auscultation in labour - In established labour, Intermittent CTG/auscultation, If FH/CTG not normal transfer to continuous CTG.	If the CTG is not normal, it must be promptly reviewed by the obstetric registrar. A plan of care must be documented on EPR
VTE risk assessment	- Complete a new VTE risk assessment if not completed within last 24 hours. - If epidural is considered, do not site until 12 hours of LMWH has been administered.	
Immediate postnatal care	- Blood pressure to be taken within an hour of delivery and document on MEOWS chart. - Aim to maintain BP <150/100mmHg - Transfer to JBW when clinically indicated - A clear plan of care must be documented in postnatal EPR record prior to transfer to ward.	- If methyldopa was used during pregnancy, stop following delivery. - The registrar (or SHO after consultation with the registrar) must document an alternative antihypertensive regime on EPR - If methyldopa was not used during the antenatal period, continue antenatal hypertensive treatment

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First presentation and outpatient management care pathway: Pre-Eclampsia

Degree of hypertension	Mild hypertension BP 140-90-149/99 mmHg	Moderate hypertension BP 150/100-159-109 mmHg	Severe hypertension BP >160/110 mmHg	Escalation to medical staff
Admit to Hospital	No unless clinical concerns.	Yes	Yes	All women to be assessed by registrar. Allocate to on call consultant on admission if not already under a consultant. Complete a VTE risk assessment
Blood pressure measurement	Three times weekly with CMW	At least four times a day	More than four times a day, depending on clinical circumstances	
Urinalysis	Do not repeat quantification of proteinuria			
Medication	Oral labetalol if BP remains above 140/90mmHg. Aim for BP of 135/85 or less			
Fetal Monitoring	Ultrasound for fetal growth (biometry), amniotic fluid volume assessment and umbilical artery Doppler velocimetry - Carry out at diagnosis if conservative management is planned if initial scan is normal repeat every 2 weeks CTG - Carry out at diagnosis - Repeat if: - Fetal movements change - Vaginal bleeding - Abdominal pain - Deterioration in maternal condition - Do not routinely repeat unless clinically indicated			If the results of any fetal monitoring are abnormal, promptly inform the obstetric registrar who should discuss with an obstetric consultant and document in the case notes
Blood tests	Twice weekly (FBC, U&E, LFT)	Three times weekly	Three times weekly	
Timing of birth	Before 34 weeks - Manage conservatively - Consultant obstetric staff to: - Document maternal (biomedical, haematological and clinical) and fetal indications for elective birth before 34 weeks - Write a plan for antenatal fetal monitoring (CTG and scan) - Offer birth if severe refractory hypertension or maternal or fetal clinical indication develops as defined in plan. 34-36+6 weeks - Recommend birth after 34 weeks if pre-eclampsia with severe hypertension and BP is controlled - Offer birth at 34- 36+6 weeks to pre-eclampsia with mild and moderate hypertension only when there is a concern about the maternal and/ or the fetal condition. After 37 weeks - The exact timing of delivery of mild/ and stable moderate pre-eclampsia should be decided between the woman and the consultant obstetrician, discussion of maternal and fetal indications for birth should be documented on EPR record			If the woman is <36 weeks gestation give a course of corticosteroids for fetal lung maturation (see preterm labour guidelines GL895). All decisions regarding delivery should be made after discussions with neonatal team

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In-patient care pathway: Pre-Eclampsia (moderate- severe)

Care		Escalation to medical staff
Complete a VTE risk assessment.	If Enoxaparin is indicated, prescribe at 22.00hrs daily.	
Blood pressure measurement	Take and record blood pressure 4 hourly daily.	If BP $\geq 140/90$ mmHg: Inform SHO, who should review the woman within 1 hour.
Urinalysis	Urinalysis is not required Repeat urine PCR quantification not required	
Blood tests	PET screen (FBC, U&E, and LFT) on day of admission. Repeat: - Twice weekly if BP $\leq 149/99$mmHg - Three times weekly if BP $> 149/99$mmHg	Clotting only if platelets $< 100,000$ Results must be reviewed on EPR.
Fetal Monitoring	CTG – on day of admission. Do not repeat if a normal CTG has already been recorded that day. Repeat CTG weekly unless the woman reports: - Change in fetal movements - Vaginal bleeding - Abdominal pain - Deterioration in maternal condition If there is concern about fetal growth the regime for CTG/scan monitoring will be decided by the obstetric team If the CTG is repeated at < 1 week then indication must be recorded on the CTG and in the woman's notes.	If the CTG is not normal it must be promptly reviewed by an obstetric registrar and may need discussion with an obstetric consultant
	Ultrasound scan for fetal growth, liquor volume and umbilical artery Doppler should be arranged within 2 days of admission. Do not repeat more frequently than every 2 weeks if normal. Ultrasound scan reports must be reviewed by obstetric registrar or consultant within 24 hours .	Borderline or abnormal results must be discussed with a consultant.
Preparation for early delivery	Administer corticosteroids and Magnesium Sulphate for neuroprotection if required in accordance with guideline GL868 . If less than 34 weeks' gestation an 'Indication for early delivery form' must be completed within 24 hrs of admission. Timing of planned delivery to be agreed by Obstetric Consultant and discussed with neonatal and anaesthetic teams.	

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Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

ANTENATAL In-patient care pathway: Pre-Eclampsia

QUICK REFERENCE GUIDE

- Complete VTE assessment and give LMWH at 22.00hrs if indicated
- 4 hourly blood pressure measurement
- Urine dipstick is not required nor is repeat PCR
- PET screen (FBC, U&E, LFT)
 - Twice weekly if BP $\leq$ 149/99mmHg
 - Three times weekly if BP $>$ 149/99mmHg
- CTG on admission and then once a week
- Only to repeat at <1 a week if the CTG is abnormal or there are changes in the woman condition e.g. she reports reduced fetal movements, vaginal bleeding, abdominal pain, deterioration in maternal condition
- Ultrasound scan – for fetal growth, liquor volume and umbilical artery Doppler should be arranged within 2 days of admission. Do not repeat more frequently than every 2 weeks if normal.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

POSTNATAL In-patient care pathway: Pre-Eclampsia

QUICK REFERENCE GUIDE

- Complete postnatal VTE assessment on EPR and give LMWH if indicated
- 4 hourly blood pressure measurement while in-patient, and then alternate days up to 2 weeks after transfer to community care, ask about symptoms at each BP check
- Aim to maintain BP $\leq$ 149/99 mmHg
- If BP $<$ 130/80 mmHg for 24hrs, reduce antihypertensive medication
- Start antihypertensive if BP $>$ 149/99 in a woman who was not on treatment
- If the woman on methyldopa during pregnancy, stop within 2 days of birth and change to labetalol or Nifedipine or ACE inhibitors
- Blood tests:
- Mild PET: do PET bloods only once at 48-72 hrs unless clinical concern
- Moderate/Severe PET: PET screen 48hrs after delivery, earlier if clinical concern, repeat as indicated if abnormal to assess improvement and finally repeat at 6-8-week postnatal check
- Postnatal stay:
 - Mild PET – 24- 48hrs
 - Moderate/Severe PET- 3-5 days, must be $>$ 24hrs since last increase in medication
- Before discharge generate a 'postnatal blood pressure management plan'
- Women with PET who took antihypertensive treatment on discharge should have BP check every 1-2 days for up to 2 weeks after transfer to community care until the off treatment and is normotensive.
- If still on antihypertensive treatment, 2 weeks after discharge will need medical review.
- Will need medical review 6-8 weeks after the birth, if still needing BP treatment will need a referral to specialist assessment of their hypertension. If still proteinuric at 6-8 weeks, will need review by GP/specialist at 3 months to assess kidney function. If kidney function deranged at 3 months, consider referring for specialist kidney assessment.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

INTRAPARTUM WARD care pathway: Pre-eclampsia		
Mild or moderate hypertension (BP ≤ 159/109mmHg)		
Care		Escalation to medical staff
Blood pressure measurement	- Hourly BP, document on partogram - If BP ≥ 160/110 Hg inform obstetric registrar - If BP stable do not routinely limit duration of second stage	If BP ≥ 160/110 Hg review woman and transfer care management to Severe pre-eclampsia and eclampsia pathway. If BP does not respond to initial treatment operative birth should be considered.
Medication	- Continue antenatal antihypertensive treatment - Use 10iu Oxytocin IM for active management of third stage	If BP ≥150/100 mmHg and no previous antenatal treatment was prescribed then antihypertensive treatment should be commenced
Urine	- If the urine dipstick on admission is 1+ or greater, arrange an urgent urinary PCR. If the woman is known to have a urinary PCR >30 mg/ml do not repeat the urine dipstick. - Follow bladder care guidelines	Registrar to review woman and document any change of plan / pathway
Blood tests	- Take blood for PET screen (FBC, U&E, LFT) - And a Group and Save on admission to labour ward unless these have been taken within last 24 hours.	Clotting screening only if platelets <100,000 Review blood results
Fetal Monitoring	Continuous CTG established labour.	Follow fetal monitoring guideline (GL964)
VTE risk assessment	- Complete a new VTE risk assessment if not completed within last 24 hours. - If epidural is considered do not site until 12 hours if LMWH has been administered	
Immediate postnatal care	- Blood pressure to be taken within an hour of delivery and recorded on MOWS chart - Aim to maintain BP <150/100mmHg - Continue antenatal antihypertensive treatment - Ask women about severe headaches and epigastric pain each time BP is measured - Women should not be transferred to Iffley ward until clinically stable. - A clear plan of care must be documented on EPR record	- If methyldopa was used during pregnancy, stop following delivery. - The registrar (or SHO after consultation with the registrar) must document an alternative antihypertensive regime in the woman's notes. - If methyldopa was not used during the antenatal period, continue antenatal hypertensive treatment

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

LABOUR WARD care pathway: Severe Hypertension, Severe Pre-eclampsia and Eclampsia

QUICK REFERENCE GUIDE

1. Do not omit oral antihypertensive treatment (unless on iv **please refer to page 31**)
2. If BP >150/100mmHg for three consecutive readings the woman must be reviewed by the Registrar.
3. If BP ≥160/110mmHg:
 - a. Take BP every 5 minutes.
 - b. Medical review.
 - c. Increase antihypertensive treatment
 - d. Continue 5 minute BP measurement until BP ≤150/100mmHg for 60 minutes then return to hourly BP measurement
 - e. If BP not ≤150/100mmHg 120 minutes after treatment – for Registrar review.
4. Consider 20% MgSO₄ infusion (**please refer to page 32/33**).
5. PET screen and group and save.
6. Urine dipstick only if pre-eclampsia not previously diagnosed.
7. Monitor and record fluid balance.
8. VTE risk assessment. Do not give LMWH in labour.
9. Anaesthetic review.
10. If <36 weeks inform NICU.
11. Continuous CTG in labour.
12. Keep NBM with 8 hourly omeprazole and cyclizine.
13. IV or IM Oxytocin for the 3rd stage of labour.
14. If BP does not respond to anti-hypertension management, consider operative delivery.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

POSTNATAL care pathway on labour ward: Severe hypertension, Severe pre-eclampsia and Eclampsia

BP ≥ 160/110 mmHg

	Care	Escalation to medical staff
Immediate postnatal care	- After delivery take BP every 30 minutes for 2 hours if BP < 150/100 mmHg reduce frequency of BP measurement to 4 hourly. These recordings must be documented on EPR. Each time the BP is checked the woman should be asked about symptoms especially headache and epigastric pain. - If BP is controlled by an infusion, continue the infusion until the registrar has reviewed the woman and documented a change to an oral regime on EPR - If the woman is on oral anti-hypertensives continue the pregnancy regime, unless the regime includes methyldopa. Methyldopa should be stopped after delivery and alternative medication prescribed. - Follow bladder care guidelines. - Continue to record fluid balance until discharge from labour ward, even after catheter is removed. Women receiving MgSO₄ Follow eclampsia pathway. - The woman will need to be observed on labour ward for at least 24 hours after MgSO₄ infusion discontinued. - Urine output should still be measured and recorded on a fluid balance chart; at this stage, a catheter is not necessary.	- The registrar (or SHO after consultation with the registrar) must document an alternative antihypertensive regime in the woman's notes. - The registrar is expected to review the woman within 4 hours of delivery and document on-going care plans. He/she will decide when transfer to Lffley ward is appropriate. - The women should be reviewed at least 8 hourly by the Labour Ward registrar who should document on-going care plans. - The labour ward registrar must document a plan for care on Lffley ward. - The registrar must review the woman between 1 and 2 hours after MgSO₄ infusion is discontinued.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

1.0 Overview

Hypertensive disorders during pregnancy occur in women with pre-existing chronic hypertension and in women who develop new-onset hypertension in the second half of pregnancy.

The rate of maternal death from pre-eclampsia and eclampsia remains low at 0.35 per 100,000 maternities in 2020-22 but there is no evidence of ongoing decrease in the mortality rate (MBRRACE 2024^(9,2)). Careful fluid management is important especially in the event of postpartum haemorrhage. Over replacement with crystalloid and / or colloid could worsen any developing coagulopathy.

There are added risk of fetal compromise when a woman's pregnancy is complicated by both hypertension and diabetes. It is not only babies predicted to be small for gestational age who may be at risk.

This guideline contains recommendations for the assessment, diagnosis and management of hypertension in pregnancy in the antenatal, intrapartum and postnatal periods in line with NICE clinical guideline NG133 (June 2019).

1.1 Definitions:-

- **Chronic hypertension** is hypertension that is present at the booking visit or before 20 weeks or if the woman is already taking antihypertensive medication when referred to maternity services. It can be primary or secondary in aetiology.
- **Gestational hypertension** is new hypertension presenting after 20 weeks without significant proteinuria.
- **Pre-eclampsia** is new hypertension presenting after 20 weeks and the coexistence of 1 or more of the following new-onset conditions:
 - 1) proteinuria (PCR over 30mg/mmol or at least 2+ on dipstick testing; **or**
 - 2) other maternal organ dysfunction:
 - renal insufficiency (Creatinine 90µmol/l or more)
 - liver involvement (ALT over 40IU/l with or without right upper quadrant or epigastric abdominal pain)
 - neurological complications (e.g. eclampsia, altered mental status, blindness, stroke, clonus, severe headaches or persistent visual scotomata)
 - Haematological complications such as thrombocytopenia / disseminated intravascular coagulation or haemolysis.
 - 3) Uteroplacental dysfunction (fetal growth restriction, abnormal umbilical artery Doppler or stillbirth).
- **Severe pre-eclampsia** is pre-eclampsia with severe hypertension (blood pressure >160/110mmHg) and/or with symptoms, and/or biochemical and/or haematological impairment.
- **Eclampsia** is a convulsive condition associated with pre-eclampsia.
- **HELLP** syndrome is haemolysis, elevated liver enzymes and low platelet count.

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Policy Lead:	Group Director Urgent Care	Version:	V6.2
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Significant proteinuria is if the urinary protein: creatinine ratio (PCR) is greater than 30mg/mmol or a validated 24-hour urine collection result shows greater than 300 mg protein per save.

Hypertension should be defined as;

- **Mild hypertension:** diastolic blood pressure 90–99 mmHg, systolic blood pressure 140–149 mmHg (**140-149/90-99 mmHg**)
- **Moderate hypertension:** diastolic blood pressure 100–109 mmHg, systolic blood pressure 150–159 mmHg (**150-159/100-109 mmHg**)
- **Severe hypertension:** diastolic blood pressure 110 mmHg or greater, systolic blood pressure 160 mmHg or greater (**≥160/110 mmHg**)

1.2 Taking the blood pressure:-

The right arm circumference must be measured and recorded in the notes (and on the MOWS chart if an inpatient). If the arm circumference is ≥35cms, the blood pressure must **always** be taken with a large cuff. If a large cuff is required, this **must** be recorded in both the woman's hand held notes and on the observation chart.

Take blood pressure using right arm, Korotkoff V sound should be used (i.e. disappearance of sound).

1.3 Urinalysis:-

Do not use first morning urine void to quantify proteinuria.

Dipstick urinalysis currently is non-automated. Any dipstick analysis in the hospital must be tested using an automated reagent-strip reading device.

If the urine analysis result is 1+ or more of protein, send a urine specimen for urinary PCR to quantify proteinuria.

Proteinuria is significant if the PCR is greater than 30 mg/mmol. If the result is greater than 30mg/mol but there is uncertainty about the diagnosis of pre-eclampsia, consider re-testing on a new sample alongside clinical review.

Do not repeat quantification of proteinuria once PET has been diagnosed.

Prognosis is not related to the extent of dipstix proteinuria.

1.4 Blood tests:-

If you request a PET screen from the laboratory (1purple top and 1gold top bottle)

They will test for– FBC, U&E's, LFT's - ALT, bilirubin, Albumin and check for clotting only if platelets count < 100,000

NICE specifically recommends that uric acid analysis is not required as part of the PET screen.

1.5 Ultrasound scan:-

NICE recommends that if a growth scan is required in a hypertensive woman the only measurements required are;

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- fetal growth
- amniotic fluid volume measurement (deepest pool in mm)
- Umbilical artery flow waveform assessment (EDF present/absent)

1.6 Management pathways:

When a woman attends the hospital with hypertension and/or proteinuria the registrar or consultant **must** indicate whether she is to follow the management pathway for:

- **Chronic hypertension**
- **Gestational hypertension**
- **Pre-eclampsia**

The agreed management pathway to be followed **must** be clearly documented on EPR and the diagnosis recorded on the EPR problem list. If the woman is an inpatient the pathway should be recorded on the hand over board/ sheet (LW and/or Iffley).

If the woman is admitted or is managed as an outpatient with moderate/severe, chronic or gestational hypertension then she **must** have a named consultant. If the woman has previously had consultant care in this pregnancy her named consultant should be recorded on the front page of her hand held and hospital notes. Woman under GP/MW care should be changed to the on call consultant for that day.

The registrar/midwife should ensure that **the correct management pathway** is being followed. If the midwife feels that the registrar is not following the correct pathway she must discuss this with the registrar and/or responsible consultant.

If the pathway for management remains unclear it is important that the registrar/midwife, contact an Obstetric consultant for a decision.

If a Consultant decides that the usual management pathway is not appropriate, then follow the Consultant's plan which must be clearly documented on EPR and reasons for deviation from RBFT guidelines must be stated. On-going management decisions in these cases must be made by the Consultant.

In-patient management for hypertension is not recommended for women with chronic or gestational hypertension unless the blood pressure is $\geq 160/110$ mmHg). Drug treatment is however recommended if the BP is $\geq 140/90$ mmHg.

Women on antihypertensive medication must not be exclusively managed in Day Assessment Unit (DAU); the woman must be given a clinic appointment at least every 2-3 weeks.

1.7 Reducing the risk of hypertensive disorders in pregnancy: -

Pregnant women should be made aware of the need to seek immediate medical advice if they experience symptoms of pre-eclampsia, including:

- Severe headache
- Visual disturbance (e.g. blurring or flashing before the eyes)
- Severe pain below the ribs

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- Vomiting
- Sudden swelling of the face/limbs

Women should be advised to take aspirin 150mg OD from 12 weeks until 36 weeks if they have either:

- One or more of the following high risk factors:
 - Pre-eclampsia or pregnancy induced hypertension during a previous pregnancy
 - Chronic kidney disease
 - Autoimmune disease (e.g. systemic lupus erythematosus, antiphospholipid syndrome)
 - Type 1 or type 2 diabetes
 - Chronic hypertension
- Two or more of the following moderate risk factors:
 - First pregnancy
 - Age 40 years or older
 - Pregnancy interval of more than 10 years
 - BMI 35kg/m² or more at booking
 - Family history of pre-eclampsia
 - Multiple pregnancy

Do not recommend the following agents/supplements to prevent hypertensive disorders in pregnancy:

- Nitric oxide donors/ progesterone/ diuretics/ low molecular weight heparin
- Magnesium/ folic acid/ antioxidants (vitamins C and E)/ fish oils or algal oils/ garlic.

Do not recommend salt restriction during pregnancy to prevent gestational hypertension or pre-eclampsia.

Give the same antenatal care advice on rest, exercise and work to women with chronic hypertension or at risk of hypertensive disorders during pregnancy as healthy pregnant women.

For women with pre-existing diabetes or gestational diabetes mellitus, please follow Diabetes in Pregnancy Guideline (GL983).

1.8 Treatment of Hypertension: -

If anti-hypertensive treatment is started the woman must be given a “Raised Blood Pressure in Pregnancy” or PET patient information leaflet. This must be documented on EPR.

BP targets are as follow:

- Chronic hypertensive:
 - <140/90mmHg in antenatal and postnatal period.
- Gestational hypertension/Pre-eclampsia:
 - Antenatal: 135/85mmHg or less

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- Postnatal: 130/80mmHg or more but less than 150/100mmHg

Before prescribing any medication check and record in the notes any current medication, history of asthma, diabetes and drug reactions.

Labetalol is the first line anti-hypertensive advised by NICE (2019) for pregnancy, provided the woman is not asthmatic, use with caution in diabetics. It should be started at a low dose (100mg BD) and increased as needed.

If a woman cannot have Labetalol or needs a second line drug NICE (2019) recommends Nifedipine or Methyldopa. Modified release Nifedipine should start at 10mg BD and be increased as needed. Methyldopa should start with a loading dose of 500mg, and then 250mg TDS then increased as needed.

2.0 Acute Management of Hypertension

If a woman has a BP >140/90mmHg recorded (using the correct size BP cuff):

1. The midwife should record the BP on the MOWS chart and ask the women about symptoms.
2. The midwife should repeat and record the BP 15 minutes later, if the BP remains >140/90 mmHg the SHO must review the woman within 1 hour. If the BP ≤140/90 mmHg - the midwife does not need to repeat the BP until it is next due on the woman's management regime.
3. CTG is only required if the woman reports abnormal symptoms or the BP is >160/110 on re-check
4. When the SHO reviews the woman he/she should take note of symptoms, drug allergies and history of asthma. He/she should also note which **management pathway** the woman is currently following but must remember that women with chronic or gestational hypertension can develop pre-eclampsia.
5. The SHO should briefly examine the woman checking for uterine or hepatic tenderness, hypereflexia and clonus. If the woman has abnormal symptoms or signs her management must be promptly discussed with a registrar.
6. A PET screen is only required if the women has abnormal symptoms or signs, or it is >3 days since the last blood test. Results must be reviewed on EPR.
7. If the woman is not currently taking any antihypertensive medication:
 - Prescribe medication to be taken immediately (100mg Labetalol if not asthmatic, or 10mg Nifedipine SR or 500mg loading dose Methyldopa if asthmatic)
 - Prescribe on-going antihypertensive medication (either Labetalol 100mg BD, or Nifedipine SR 10mg BD). or methyldopa 250 mg TDS This regular prescription must be given within 12 hours of the first dose of antihypertensive medication.
 - The blood pressure should be checked and recorded 1 hour after giving the first dose of medication.
 - If BP ≤140/90 mmHg repeat BP as per management pathway.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- If BP >140/90 mmHg repeat BP measurement 1 hour later (this will be 2 hours after medication). If still >140/90 mmHg a further dose of labetalol or Nifedipine SR or methyldopa can be given 2 hours after the first dose following a discussion with the registrar, who must also review the woman within the next hour. **If a second dose** is needed the woman will need **transfer to the labour ward**, and CTG should be considered (regardless of symptoms and signs).
 - Repeat BP 1 hour (and if needed 2 hours) after the 2nd dose of medication. If the BP is still >140/90 mmHg 2 hours after the 2nd dose of medication the woman's management **MUST** be discussed with a consultant. Parenteral antihypertensive medication should be considered (management pathway and regimes are given in the severe hypertension, severe pre-eclampsia on labour ward section of this guideline).
8. If the woman is currently prescribed antihypertensive medication:
- Check notes and follow suggested registrar or consultant plan.
 - If no recorded plan:
 - Give an extra tablet of labetalol (100mg) or Nifedipine SR (10mg) or Methyldopa (250-500mg) immediately **AND** increase regular antihypertensive medication. Generally, the medication will be doubled e.g. increase labetalol 100mg bd to 200mg bd, increase Methyldopa 250mg to 500mg TDS or Nifedipine SR 10mg BD to 20mg BD.
 - The BP should be checked and recorded 1 hour after giving the extra dose of medication. **If a second dose** is needed the woman will need **transfer to the labour ward**, and CTG should be considered (regardless of symptoms and signs).
9. The SHO must discuss his/her management with a registrar or consultant and must document this discussion on the EPR.
10. It is important that the patient's blood pressure and medications are reviewed on the medical rounds.

3.0 Management of antenatal inpatients with hypertension

At the presentation use the flowchart on page 6 to select the most suitable management care pathway: -

3.1 Antenatal inpatient Care Pathway: Severe Chronic Hypertension (page 8)

- 1) Admission is only required to control the blood pressure if >160/110mmHg.
- 2) Once the BP is ≤140/90mmHg for 24 hours a woman with chronic hypertension can be discharged home, but with clear follow up arrangements in ANC or DAU.
- 3) Follow the chronic hypertension inpatient care pathway
- 4) If the woman is <36 weeks' gestation give a course of corticosteroids for fetal lung maturation as per [RBH guideline GL920](#)

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- 5) Delivery before 37 completed weeks is rarely required in women with chronic hypertension. If, however, the hypertension is refractory it may be considered. This decision must be made by a consultant obstetrician. If early delivery is planned arrange NICU visit and review by the neonatal team.
- 6) Remember women with chronic hypertension can develop superimposed pre-eclampsia. If this occurs the management should then follow the pre-eclampsia pathway.

3.2 Antenatal in-patient care pathway: Gestational Hypertension (page 13)

- 1) Admission is required to control the blood pressure if >160/110mmHg. Once the BP is <140/90mmHg for 24 hours a woman with gestational hypertension can be discharged home, but with clear follow up arrangements in ANC or DAU.
- 2) Admission is also required if there are biochemical/haematological concerns/impending eclampsia/impending pulmonary oedema/suspected fetal compromise.
- 3) Follow gestational hypertension inpatient care plan
- 4) Administer a course of corticosteroids in accordance to the [Steroid Prophylaxis against RDS Guideline GL920](#)
- 5) Delivery before 37 completed weeks is rarely required in women with gestational hypertension. If, however, the hypertension is refractory it may be considered. This decision must be made by a consultant obstetrician. If early delivery is planned arrange NICU visit and review by neonatal team.

3.3 Antenatal care after discharge:

- The woman's care will now be hospital based. All appointments will now be in the DAU or ANC.
- Twice weekly BP and urinalysis
- Weekly PET screen and review by registrar or consultant with results
- CTG not required if BP controlled and woman reports good fetal movements.
- USS only if clinically indicated.

3.4 Antenatal Inpatient care pathway: Pre-eclampsia (page 17)

- 1) Women with pre-eclampsia and a PCR of 1 g /mmol or > (+2) should be admitted regardless of the severity of hypertension. The woman will then remain an inpatient until after she has given birth.
- 2) Follow pre-eclampsia in-patient care pathway.
- 3) Administer corticosteroids in accordance to the [Steroid Prophylaxis against RDS GL920](#). Within 24 hours of admission the consultant obstetrician responsible for any woman admitted with pre-eclampsia should document the 'indication for early delivery' on patient's EPR record ([see appendix 1](#)). This will indicate when delivery before 34+0 should be considered.
- 4) Timing of Delivery:

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

1. Aim to manage women with pre-eclampsia conservatively until 34+0 weeks
 2. IOL or planned caesarean (as clinically appropriate) could be considered for women with pre-eclampsia after 37+0 weeks. This must be agreed with a consultant; this should be documented on EPR.
- **Women with Pre-eclampsia and mild or moderate hypertension (159/109mmHg or below)**
 IOL or planned caesarean section (as clinically appropriate) can be offered after 37 weeks depending on maternal and fetal condition, risk factors and neonatal availability.
 Plans must be agreed with an obstetric consultant before discussion with the parents.
 - **Women with pre-eclampsia and severe hypertension (160/110mmHg or higher)**
 Consider IOL or planned caesarean after 34 weeks once blood pressure has been controlled and a course of corticosteroids, if appropriate, has been completed. Do not use steroids as treatment for HELLP.
 Plan of management must be agreed with a consultant and discussed by a consultant or registrar with the parents; this should be recorded on EPR.
 Timing of delivery must be discussed with neonatal and anaesthetic teams; this should be recorded on EPR.

4.0 Intrapartum care

4.1 Mild or moderate hypertension (BP 140/90-159/109 mmHg)

Women with hypertension should have normal Intrapartum care in line with 'Intrapartum care: management and delivery of care to women in labour' (NICE clinical guideline 55) in conjunction with the care pathways below.

- 1) Follow the appropriate labour ward care pathway
- 2) Continue antenatal antihypertensive treatment (if any) during labour.
- 3) Take and record the blood pressure hourly and document on the partogram.
 - If BP $\geq$ 160/110 mmHg, registrar to review woman and transfer care management to **Severe hypertension care pathway**.
- 4) Follow the bladder care guidelines for labour.
- 5) If urinalysis shows an unexpected 1+ or more protein:
 - PET screen
 - Woman to be examined by the registrar

Author:	S Wong, S Williams	Date:	March 2026
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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- 6) In women known to have pre-eclampsia check flow chart and repeat PET screen if more than 24 hours since last test.
- 7) Fetal monitoring
 - Pre-eclampsia – continuous CTG in established labour
 - Chronic Hypertension or Gestational hypertension with mild or moderate hypertension, CTG on admission for a minimum of 30 minutes. If CTG trace is normal, intermittent CTG/auscultation to be carried out in labour. If CTG trace is not normal CTG monitoring should be continuous
 - If there are concerns about fetal growth the woman should have a continuous CTG once in established labour
- 8) Complete a VTE risk assessment (if not already completed).
 - Do not give low molecular weight heparin (LMWH) during labour.
- 9) Do not routinely limit the duration of the second stage of labour.
- 10) Use 10iu Oxytocin IM (or IV) for active management of the third stage.

4.2 Immediate postnatal care on the labour ward

Blood pressure to be taken within an hour of delivery and repeated 4 hourly. This should be documented on EPR.

1. Women with pre-eclampsia should be asked about severe headache and epigastric pain each time BP is measured. This should be documented on EPR.
2. If Methyldopa was used during pregnancy this should be stopped and changed within 2 days. The registrar (or SHO after consultation with the registrar) must document an alternative antihypertensive regime in the woman's notes.
3. If methyldopa was not used during the antenatal period, continue antenatal antihypertensive treatment.
4. Aim to maintain BP <150/100mmHg.
5. Transfer to Iffley ward when clinically stable and suitable for transfer.
6. A clear plan of care must be documented on EPR

5.0 Management of Severe Hypertension and Severe Pre-eclampsia / eclampsia on labour ward ([page 21](#))

5.1 Overview:

A serious, life-threatening, multisystem disease affecting the mother and fetus. Successful management requires a multi-specialty team approach with direct senior input to achieve urgent delivery after stabilisation.

5.2 Definitions

- **Eclampsia:** one or more epileptiform fits in a pregnant, or recently delivered woman, in association with clinical or biochemical pre-eclampsia

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- *Severe (fulminating) pre-eclampsia*: DBP $\geq$ 110 mm Hg, SBP $\geq$ 160 mm Hg and proteinuria $> 2+$ on 2 occasions

Or

- Signs and/or symptoms of imminent eclampsia i.e. persistent frontal headache, visual disturbances, epigastric tenderness, hyper-reflexia and evidence of any renal, hepatic or haematological impairment.

A red Eclampsia box containing all the necessary drugs and equipment is stored on top of the crash trolley on the Delivery Suite, Level 4 and Rushey ward, Accident and Emergency Department and Theatre.

5.3 Anti-hypertensive therapy:

Continue use of antenatal antihypertensive treatment during labour.

If blood pressure becomes unstable consider treatment with one of the following parenteral antihypertensive treatments and stop the oral antihypertensive treatments.

5.3.1 *Hydralazine*

- Slow intravenous bolus of 5-20 mg (20mg hydralazine in 20 ml 0.9% NaCl) as slow bolus over 10 – 20 minutes for immediate control.
- Hydralazine maintenance infusion- Hydralazine 60 mg in 60 ml 0.9% NaCl (1mg/ml) administered by pump at 1-12 ml/hr (1-12 mg/hr) titrated against diastolic blood pressure. (The side effects of Hydralazine are tachycardia, headache, vomiting and tremor)

Consider 500ml crystalloid fluid prior or during administration of antenatal hydralazine.

If not already on oral antihypertensive treatment, it should be commenced when iv treatment has been discontinued.

5.3.2 *Labetalol*

- 200mg per oral stat (prior to or in absence of iv access) or IV 50mg bolus slowly over 5 minutes, increase bolus by 40-80 mg every 10 minutes to max of 200mg.
- Labetalol maintenance: 100mg Labetalol in 100ml 5% glucose and administer at a rate of 20ml/hour, doubling every 30 minutes to a max of 160ml/hr until BP control is achieved. Consider double strength solution (200 mg in 100 ml) if BP not controlled.
NB: Labetalol is contraindicated in asthma, bradycardia and pulmonary oedema. Use with caution in diabetics.
- Nifedipine: 10 mg orally, repeated once, after 30 min if BP not adequately controlled ($\geq 160/110$ mmHg) commence either IV labetalol or IV hydralazine - starting with bolus dose first.

If these measures fail to control the BP and other pharmacological agents have to be administered, the patient should be transferred to ICU following delivery.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Take and record bloodpressure (BP) **every 5 minutes** using an automated BP machine to monitor response to treatment and to ensure BP stabilising, then check BP at 15 min intervals using automatic BP machine and manually once every hour using appropriate sized BP.

Once the BP has been ≤140/90mmHg for 60 minutes return to measuring and recording BP hourly. Remember to document all the readings on EPR Vital signs.

1. Keep nil by mouth, give omeprazole and cyclizine as per guideline. Commence iv fluids unless delivery in next 12hrs is not considered.
2. Take blood for PET screen and a Group and Save on admission to labour ward. Take a PET screen every 6-12 hours, a clotting screen is required only if there is concern about platelet count. Ensure all results are recorded on EPR clearly documenting time bloods taken.
3. If PET diagnosis was not confirmed prior to this admission test urine for proteinuria if urine dipstick on admission is 1+ or greater this confirms the diagnosis and urgent urinary PCR is needed unless delivery is imminent. If the woman is known to have a urinary PCR >30 mg/ml do not repeat the urine dipstick.
4. Continuous electronic fetal monitoring must be commenced. Follow fetal monitoring guideline.
5. Record fluid balance carefully, all IV fluids should be administered via a pump
 - If a catheter is in situ record urine output hourly, if not catheterised measure and record each void.
 - Limit maintenance fluids 120ml/hr in labour, 80mls/hour if antenatal or postnatal. Reduce or stop iv fluids if drinking.

5.4 Anticonvulsants

Consider the use of MgSO₄ if a woman has severe PET ([see below](#)) the registrar should discuss this decision with the on call consultant. The outcome of the discussion must be recorded on EPR. The labour ward shift leader must be informed.

Severe hypertension (BP ≥160/110 mmHg) or mild or moderate hypertension and proteinuria with at least one of the following;

- Severe headache
- Visual disturbances
- Severe pain below ribs or vomiting
- Papilledema
- Clonus (>3beats)
- Liver tenderness
- HELLP syndrome
- Platelet count <100X10⁹/l
- ALT or AST >70iu/l

Author:	S Wong, S Williams	Date:	March 2026
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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

If a woman has an eclamptic fit start MgSO₄ infusion. Do not use diazepam/phenytoin or other anticonvulsant in women with eclampsia.

5.4.1 Immediate management of an eclamptic fit and magnesium sulphate infusion

(MgSO₄ BOX is kept on bottom drawer of the emergency trolley on Delivery suite)

1. Call for help using emergency bell. Do not leave woman alone.
2. Secure airway, place patient in left lateral position, and administer oxygen. Ensure resuscitation equipment nearby.
3. Ring 2222 and ask for 'Obstetric emergency', call LW coordinator to room if not already present.
4. Establish IV line (take 20 ml blood).

Magnesium Sulphate Preparation (please note the new 20% MgSO₄ preparation is a ready mix and does not need any dilution, it comes in a 10 ml small bottle (vial).

Take 2 separate syringes:

Syringe 1 (20 ml syringe): Loading dose = 4g (16mmol) Magnesium Sulphate by slow IV bolus over 5 – 10 minutes.

Take ONE 20 ml syringe and fill with the contents of TWO 10ml ampoules of 20% Magnesium Sulphate. This contains 4g (16mmol) of Magnesium Sulphate. Give the 4g (16mmol) Magnesium Sulphate by slow IV bolus, over 5-10 minutes.

Syringe 2 (50 ml syringe): Maintenance infusion = 1g/hour Magnesium Sulphate for at least 24 hours after initiation, or after birth, or after last eclampsia, whichever is the latest.

Take FIVE 10ml ampoules of 20% Magnesium Sulphate and put it in a 50ml syringe. Give via a Syringe Driver 5ml/hour for 24 hours.

Monitor the urine output:- **If the woman is ANURIC, only the loading dose may be given**

5.4.2 Further seizures while on magnesium sulphate infusion

If further seizures occur:

- Seek immediate senior help from the on-call obstetric and anaesthetic registrar and inform the consultant obstetrician and consultant anaesthetist
- **A further single bolus of 2g may be given, following the instructions below: Take ONE 10ml syringe:- Draw up 10mls of the ready diluted 20% Magnesium Sulphate. This contains 2g - give at a rate of 2mls per minute IV over 5 minutes manually. Do not repeat**
- If possible take blood for magnesium level (gold blood bottle) prior to giving the bolus dose

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

5.4.3 Monitoring during MgSO₄ therapy

Every 15 minutes during first two hours of therapy and hourly thereafter if condition stable, until stopped on consultant obstetrician review

- Continuous ECG and pulse oximetry monitoring throughout O₂ saturation and pulse
- Blood pressure
- Patellar reflexes (or biceps if there is a functioning epidural)
- Respiratory rate
- Conscious level
- Hourly urine output
- Do not routinely check Magnesium Sulphate level unless signs of Magnesium Sulphate toxicity are present (section 5.4.4 below).

5.4.4 Magnesium Sulphate Toxicity

If any of signs below are present, **stop MgSO₄ infusion and request immediate medical review**

1. Urine output <100ml in 4 hours. If there are no other signs of toxicity consider reducing the Magnesium infusion to 0.5g/hr.
2. Absent patellar reflex - if respiration normal (more than 10 breaths per minute) stop Magnesium Sulphate infusion until the reflexes return
3. Respiratory depression (less than 10 breaths per minute) give O₂ by facemask, stop Magnesium Sulphate infusion, give 10mls, 10% calcium gluconate given by slow intravenous injection over 5-10 minutes. Maintain airway and nurse in the recovery position.
4. Respiratory arrest - intubate and ventilate, stop Magnesium sulphate therapy. Give 10mls 10% calcium gluconate IV over 5-10 minutes. Continue ventilation until spontaneous breathing recurs

5.4.5 Monitoring the Dosage of Magnesium Sulphate, (MgSO₄)

Level of Magnesium Sulphate	Range (mmol/litre)	Action
Therapeutic	2.0 - 3.5	
High	3.55 – 5.0	Stop infusion for 15 minutes Restart at half the previous rate if urinary output ≥20 ml/hr Recheck blood level one hour after the infusion was temporarily stopped If the urine output <20mls ask for Obstetric Consultant advice before restarting infusion

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Very High	> 5.0	Stop infusion Ask for Obstetric Consultant advice urgently
Low	< 2	Increase rate of infusion to 10mls/hour (equivalent to 2g/hr) for 2 hours only Recheck Magnesium Sulphate level 3 hours post increase

- Send blood for:
 - PET screen
 - Clotting screen
 - Group and Save
- Document time blood is taken on EPR.
 5. The on call obstetric consultant must be informed of events and asked to attend. If there are any problems with airway management/central line or if C/S planned, the labour ward anaesthetist must discuss the management with the on-call Consultant anaesthetist.
 6. Insert urinary catheter - for hourly urine output measurement.
 7. Start input / output chart, this must be accurate as a decrease in urine output may indicate a need for change in the management plan. Test urine for protein if pre-eclampsia not formerly diagnosed. Urinalysis for protein is not required if the woman is known to have pre-eclampsia.

5.5 Blood pressure:

If BP $\geq$ 160/110 mm Hg manage as for severe hypertension. Note that oral drugs may not be suitable if postictal (drowsy). Intramuscular injections are contra-indicated if the platelet count is $< 100 \times 10^9/l$. If hydralazine or Labetalol infusion is required ensure appropriate decrease in infusion rate of IV fluids.

5.6 O2 saturation levels:

This should remain above 97%. If levels fall below this check respiratory rate every 15 minutes, inform labour ward obstetric registrar who should listen to the chest. If there is any evidence of pulmonary oedema arrange chest x-ray then if confirmed give 20mg intravenous furosemide. If there is evidence of pulmonary oedema on the x-ray management must be discussed with both the obstetric and anaesthetic consultants.

5.7 Fluid Balance:

In severe pre-eclampsia there is severe intravascular depletion and a contracted vascular bed. This means that responses to fluids may be atypical and difficult to assess. Consequently, great care with fluid balance is required as there is a real danger of fluid overload.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

The following guidelines should be observed:

- Replace obvious blood loss at delivery
- Then **fluid restrict** to maintain total fluid input at 40 ml per hour + previous hour's urine output, given as crystalloid (plasmalyte) to a maximum of 80ml per hour
- Do not chase a 'satisfactory' urine output. The patient is liable to develop pulmonary oedema. Irreversible renal damage is unlikely after a short period of oliguria secondary severe pre-eclampsia.
- A central line is rarely indicated unless there has been a major obstetric haemorrhage or concerns about cardiac function. Check clotting before insertion
- SpO2 deterioration below 95% may indicate impending pulmonary oedema. A doctor should perform auscultation of the chest.
- Diuretics are only used in confirmed pulmonary oedema after discussion with the on-call consultant obstetrician
- Do not preload women who have severe pre-eclampsia with intravenous fluids before establishing low-dose epidural analgesia or combined spinal epidural analgesia.
- Do not use volume expansion in women with severe pre-eclampsia unless hydralazine is the antenatal antihypertensive.
- **Fluid restrict** until stopped by a consultant obstetrician

5.8 Urine output:

- If urine output is low (<100ml/ 4 hours) carefully assess fluid balance
- Repeat PET screen
- If creatinine >120mmol/l the management must be discussed with the on call consultant. The discussion must be recorded on EPR.

5.9 Timing of delivery:

- If the woman has an eclamptic fit, she should be stabilised and then both mother and fetus assessed for mode and timing of delivery. This will depend upon the gestation of the fetus.
- In severe pre-eclampsia, pregnancy should not be prolonged to gain fetal maturity at the expense of deteriorating maternal condition.
- The decision to deliver and mode of delivery will be made a senior obstetrician in consultation with the neonatal and anaesthetic staff, following review of the biochemistry, maternal observations & condition and gestation of fetus.
- If the fetus is alive caesarean section is usually appropriate unless vaginal delivery is imminent. Continuous CTG monitoring until delivery is mandatory.
- If the fetus is alive the neonatal team should be informed of the eclamptic fit, its management and plans for birth. A paediatrician should be called to attend the birth even if fetal compromise is not suspected.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- If the fetus has died vaginal delivery is most appropriate provided it can be achieved within 12 hours of the eclamptic fit.

5.10 Analgesia

- Providing the clotting is normal **and** the platelet count $> 80 \times 10^9/L$ **and** there are no other contra-indications consider an epidural for analgesia in labour, for Caesarean section and post operatively for analgesia. Use colloid/crystalloid **carefully** for co-loading. 500 ml to 1000 ml will be sufficient.
- Take care with narcotics. These patients have a tendency to respiratory depression.
- If epidural analgesia is not possible then consider PCA rather than IM bolus administration.

5.11 Coagulation control

- If there is an abnormal clotting profile or low platelet count i.e. $< 80 \times 10^9/L$ prior to a surgical procedure, or there is clinical DIC, seek the advice of the Consultant Haematologist on-call.
- The patient may require platelet concentrate and/or fresh frozen plasma and cryoprecipitate transfusion.

5.12 Anaesthesia for delivery

- Discuss with senior anaesthetic staff before commencing anaesthetic. If an eclamptic fit has occurred the decision about mode of anaesthetic, should be made by a consultant anaesthetist.
- Providing the woman is alert and oriented and a platelet level above $80 \times 10^9/L$ then a regional technique can be considered.

However, if

- She remains confused
- Has rapidly evolving neurological signs
- Evidence of falling platelets or disseminated intravascular coagulation (DIC) then a general anaesthetic should be performed

If giving a general anaesthetic consider the following

- Beware of laryngeal oedema causing difficult intubating conditions
- Beware of pulmonary oedema
- Consider giving an opioid (Alfentanil, Fentanyl or Remifentanyl) prior to induction. Warn paediatrician that opioids have been used
- Give generous induction dose of Thiopentone
- Avoid Diclofenac and other NSAIDs in view of impaired renal function
- Pain relief can be difficult so give IV Paracetamol and consider PCA morphine
- Magnesium Sulphate will prolong the action of all muscle relaxants

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

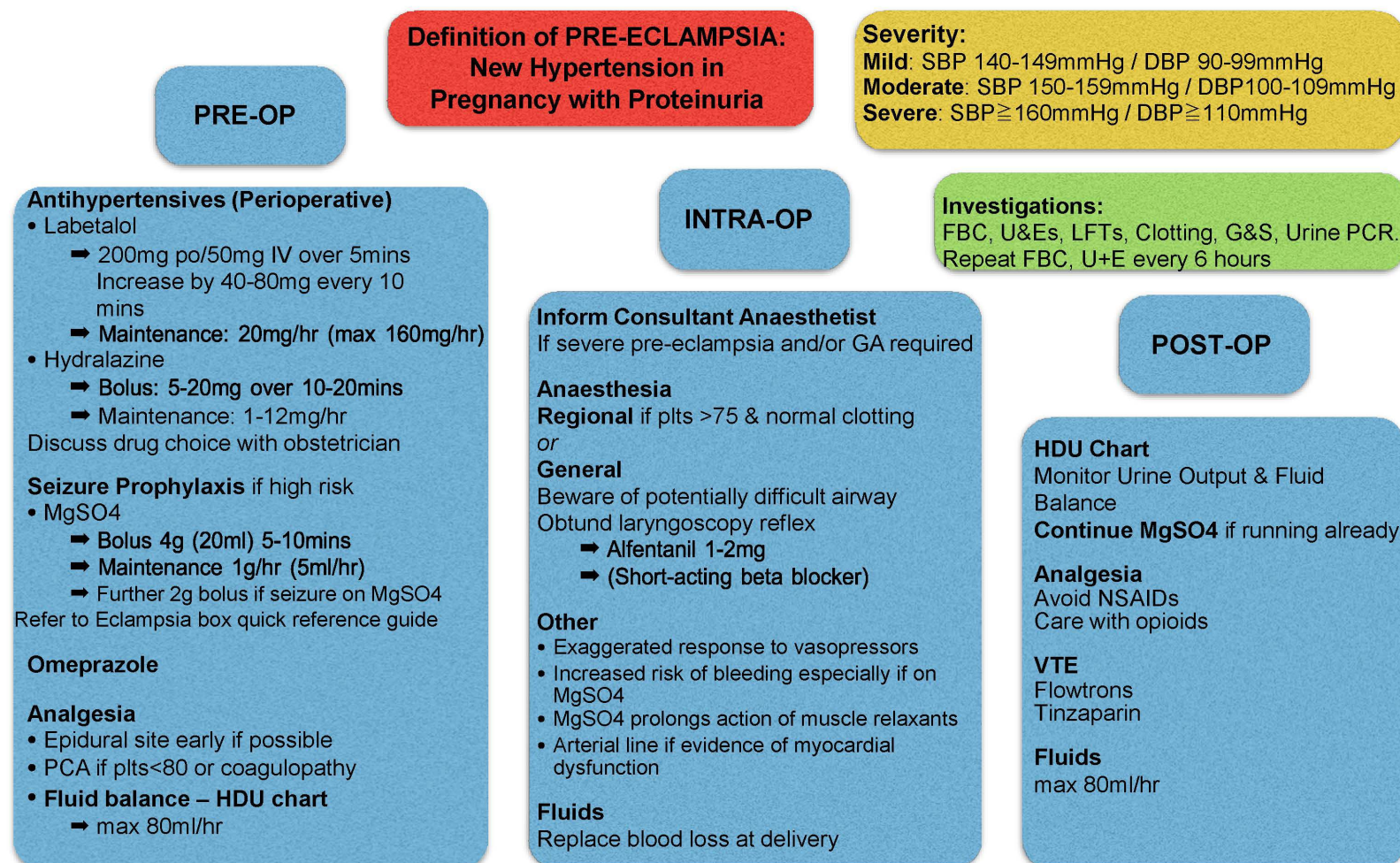
especially non-depolarising blocking agents. Use Suxamethonium and then either avoid the non-depolarising agents or use a reduced dose. Use a nerve stimulator. Do not attempt extubation unless satisfactory return of respiratory function and muscle tone.

- They may have an abnormally exaggerated cardiovascular response to vasopressor drugs.
- These women will be at an increased risk of post-partum haemorrhage, particularly if on a Magnesium Infusion, if Carbetocin is used, an Oxytocin infusion should not be used for at least 4 hours. All other uterotonics may be used to control a postpartum haemorrhage.
- Consider using an arterial line if there is evidence of myocardial dysfunction

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

EMERGENCY MEDICAL ALGORITHM (EMA-080)

GUIDANCE ON PERI-OPERATIVE ANAESTHETIC MANAGEMENT OF PRE-ECLAMPSIA*



*Taken from Maternity Guideline GL952

Written by Dr Gopalan Radhakrishnan 2016
Reviewed by Dr S Williams/ Ms S Wong,
February 2025

Policy Lead:	Group Director Urgent Care	Version:	December 2025 V6.2
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

5.13 Post-partum

It is common for the clinical and biochemical aspects of pre-eclampsia to deteriorate in the 24 hr after delivery.

Complete a new VTE risk assessment if not completed within last 24 hours. Do not give LMWH during labour but start in the immediate postnatal period when it is safe to do so. Use flowtrons until she can be given her first postnatal dose of LMWH.

Physiotherapy: daily physiotherapy for prevention of DVT and chest infection in the puerperium till mobile.

6.0 POSTNATAL Labour Ward care pathway: Severe Hypertension, severe pre-eclampsia and Eclampsia (page 20/21)

6.1 Immediate postnatal care of women who have received MgSO₄

1. The mother should remain on labour ward until review by an experienced obstetrician (ST4 or higher) 2 hours after MgSO₄ is discontinued. Maternal observations and frequency of these should be as below. If there are no on-going concerns transfer to a post-natal bed should be arranged. Any mother showing signs of persistently brisk reflexes, or other signs of cerebral irritation should not be transferred at this time but subjected to continued review and discussion with the consultant on duty.
2. Urine output should be measured and recorded on a fluid balance chart; at this stage, a catheter is not necessary. Follow bladder care guidelines
3. After discontinuing MgSO₄ take BP every 30 minutes for 2 hours if BP <140/90 mmHg reduce frequency of BP measurement to 4 hourly. These recordings must be documented on vital signs on EPR. Each time the BP is checked the woman should be asked about symptoms especially headache and epigastric pain.
4. The woman should be reviewed by the registrar between 1 and 2 hours after the MgSO₄ infusion has been stopped.
5. The woman should later be reviewed at least 8 hourly by the labour ward registrar who should document on going care plans

6.2 Immediate postnatal care on the labour ward of women with severe hypertension and/or eclampsia.

1. If on MgSO₄ follow MgSO₄ guidelines.
2. After delivery take BP every 30 minutes for 2 hours if BP <150/100 mmHg reduce frequency of BP measurement to 4 hourly. These recordings must be documented on vital signs on EPR. Each time the BP is checked the woman should be asked about symptoms especially headache and epigastric pain.
3. If BP is controlled by an infusion, continue the infusion until the registrar has reviewed the woman and documented a change to an oral regime on EPR.
4. If the woman is on oral antihypertensive continue the pregnancy regime, unless the regime includes methyldopa. Methyldopa should be stopped after delivery;

Author:	S Wong, S Williams	Date:	March 2026
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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

the registrar (or SHO after consultation with the registrar) must document an alternative antihypertensive regime in the woman's EPR record.

5. Follow bladder care guidelines.
6. Continue to record fluid balance until discharge from labour ward, even after catheter is removed.
7. Complete postnatal VTE risk assessment. If postnatal LMWH is required the registrar should decide when the first dose can be given – this will depend on clinical circumstances, the platelet count and renal function. If the postnatal dose cannot be given within 6 hours of delivery the woman should have flowtrons fitted until the first dose of LMWH is given.
8. The use of NSAID's for pain relief may be contraindicated in women with pre-eclampsia. The registrar should decide (and document) if/when NSAID's can be given
9. The registrar is expected to review the woman within 4 hours of delivery and document on-going care plans. He/she will decide when transfer to Iffley ward is appropriate.

6.3 Postnatal care pathway on labour ward:

See [page 21](#)

7.0 In-patient postnatal care

7.1 Postnatal ward management of hypertensive women

Ideally a postnatal plan for anti-hypertensive medication will have been documented during the antenatal period. If this has not been done then antihypertensive medication must be reviewed by the Labour Ward registrar before the woman is transferred to the ward. At the time of transfer to the post-natal ward the woman's notes must clearly indicate whether she is to be managed on the chronic hypertension, gestational hypertension or pre-eclampsia pathway. If the labour ward midwife is not sure this must be clarified and documented on EPR by the labour ward registrar.

1. Methyldopa should be stopped within 2 days after delivery and alternative medication prescribed.
2. Women with chronic hypertension should continue their pregnancy regime after delivery.
3. Advise women who are on antihypertensive that their antihypertensive treatment can be adapted to accommodate for breastfeeding.
4. Inform women who wish to breastfeed and are on antihypertensive that:
 - Antihypertensive medications can pass into breastmilk in very small quantity and unlikely to have any clinical effect

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

- Most medicines are not tested in pregnant or breastfeeding women; hence disclaimers information are not because of any specific safety concerns or evidence of harm.
5. Consider monitoring BP of babies, especially those born preterm, who have symptoms of low BP for the first few weeks.
 6. When discharge home, advise women to monitor babies for signs of drowsiness, lethargy, pallor, cold peripheries or poor feeding.
 7. Where possible try to use antihypertensive that are only taken once daily.
 8. Offer enalapril to treat hypertension in postnatal period with adequate monitoring of maternal renal function and serum potassium. [See Table 9 \(page 57\)](#).
 9. For women of black African or Caribbean family origin, consider
 - nifedipine or
 - amlodipine if previously used this with successful BP control
 10. If BP not controlled with single agent antihypertensive, consider either:
 - Adding atenolol or labetalol or
 - Swapping 1 of the medicines being used for atenolol or labetalol
 11. Avoid diuretics and angiotensin receptor blockers to treat hypertension in women who are breastfeeding or expressing milk.
 12. Treat women with hypertension in postnatal period who are not planning to breastfeed in line with the NICE guideline on hypertension in adults (<https://www.nice.org.uk/guidance/cg127>)

7.2 Postnatal blood pressure management:

13. Women with hypertension should have their blood pressure monitored four hourly on the post-natal ward. Women with pre-eclampsia should be asked about epigastric pain and headache each time their blood pressure is measured. All women with hypertension should also be reviewed by a doctor each day
14. Women with chronic hypertension, gestational hypertension or mild pre-eclampsia can be discharged after 48hrs if symptom free and blood pressure is controlled. Women with severe or moderate pre-eclampsia should remain in hospital for 3-5 days

7.3 Maintenance of blood pressure:

- Chronic hypertension: aim to maintain BP at, or below 140/90mmHg.
- Gestational hypertension and pre-eclampsia: aim to maintain blood pressure at or below 149/99mmHg.
- Reduce antihypertensive treatment if the blood pressure falls below 130/80mmHg for >24 hours
- If BP $\geq$ 150/100mmHg increase antihypertensive medication.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

15. If antihypertensive medication is increased then the woman should stay in until her blood pressure has been satisfactory for 24 hours, or a consultant review has taken place and discharge is agreed.
16. If anti-hypertensive treatment is started for the first time the woman must be given a “Raised Blood Pressure in Pregnancy” patient information leaflet. This must be documented on EPR.
17. If BP $\geq$ 150/100mgHg the midwife should document this on the four hourly MOWS observation chart and repeat after 15 minutes if still $\geq$ 150/100mgHg call the ward SHO to review the woman. The SHO is expected to see the woman within one hour and should start or increase the woman’s antihypertensive medication.
18. The doctor should review the patient, with particular attention to any symptoms, hepatic tenderness, increased reflexes and/or sustained clonus.
19. Before prescribing any medication note should be taken, and recorded, of current medication, history of asthma and drug reactions.
20. The SHO must always discuss their findings and treatment with the duty registrar, (this must be documented on EPR by the SHO). If the woman has abnormal symptoms or signs this discussion must be prompt, and a decision made about whether blood tests, review by registrar or transfer to Labour Ward is required.
21. If the woman has increased blood pressure, but no abnormal symptoms or signs, she can be managed on the postnatal ward. Her anti-hypertensive medication will need to be increased, after discussion with a registrar or consultant.
 - A suggested anti-hypertensive drug plan may have been recorded in the woman’s EPR record by the obstetric team. If so, please follow.
 - Women with postnatal hypertension are usually managed with labetalol and/or Nifedipine. Use of other drugs must be discussed with a consultant.
22. Women with chronic hypertension, gestational hypertension or mild pre-eclampsia do not require postnatal PET blood tests unless they develop abnormal signs, symptoms or have very erratic blood pressure measurements.
23. Women with Pre-eclampsia
 - PET screen at 48 hours
 - If results are normal, do not repeat.
 - If results are abnormal or not improving plans for future tests must be made by a registrar probably in consultation with a consultant.
24. If the drug regime is changed remember to amend the TTO prescription. Women should be prescribed 2 weeks of their antihypertensive medication.
25. Before discharge from the ward the midwife must clearly document in the problem list on EPR whether the woman has chronic hypertension, gestational hypertension or pre-eclampsia. The community midwife will need this information for on-going management.

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

26. Before discharge from the ward the SHO must update the GP discharge letter on EPR to include the “postnatal blood pressure management plan” for her on-going community care ([see appendix 2](#)).
27. Arrange postnatal medical review in hospital or inform the patient to arrange with GP in 6-8 weeks’ time.

8.0 Postnatal care following discharge from hospital

8.1 Women with Chronic Hypertension

These women will usually be discharged home two days after giving birth.

They will stay on anti-hypertensive medication long term.

The community midwife should check the blood pressure on day 4.

- If the blood pressure is <140/90 mmHg and the woman does not complain of dizziness/fainting then the midwife does not need to arrange any further BP checks. The woman should arrange a BP review with her GP at two weeks, when she will need to get her on-going prescriptions.

The community midwife must ensure that the woman’s current antihypertensive regime is clearly documented on her EPR record. The woman should be reminded to take this care plan with her when she has her two week BP review with her GP. The community midwife will need to collect the care plan from the woman after her GP review.

- If the blood pressure is 141/91 -150/100mmHg the midwife should check the BP two days later. Then manage as above.
- If the BP is >150/100mmHg the midwife should phone the midwife in charge of the antenatal clinic, while she is with the patient (out of hours LW shift leader), for advice. Women with chronic hypertension do not need to be re-admitted to hospital unless there are other concerns. A change in antihypertensive treatment is likely to be advised. If the woman has her medication changed the community midwife will check her BP one day later and follow the above guidance.

8.2 Women with Gestational Hypertension

All women with gestational hypertension, even if not on medication, must have a BP check on day 4. Women on medication should have alternate day BP checks with the community midwife until off medication. If the woman is still on medication 12 days after delivery she must be told to arrange an appointment with her GP on day 13/14. Her GP should then manage her medication and will be responsible for prescribing any on-going anti-hypertensive medication.

If at any check the woman has raised blood pressure >149/99mmHg arrangements must be made for her to be reviewed at the hospital usually DAU (LW out of hours).

Author:	S Wong, S Williams	Date:	March 2026
Job Title:	Consultant O&G, Consultant Anaesthetist	Review Date:	December 2026
Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

The woman will have been given a postnatal BP management plan when discharged from the ward. When the community midwife checks her blood pressure she/he should reduce the woman's anti-hypertensive medication according to this plan until the BP is <130/80mmHg, and she is off all antihypertensive medication. If the woman is still on anti-hypertensive medication on day 12, the community midwife must ensure that the woman's current antihypertensive regime is clearly documented on her EPR record.

8.3 Women with Pre-Eclampsia

All women with pre-eclampsia can be discharged to community care if BP remains below 150/100mmHg (with/without anti-hypertensive)/ no symptoms of PET and blood results are stable/improving.

Women with mild pre-eclampsia (unlikely to be on medication) will be discharged home from hospital on day 2. The community midwife should take a blood pressure and check for symptoms on day 3, 4 and 6. If the woman is symptom free and the BP is <150/100 mmHg no action is required. If the woman has raised blood pressure or symptoms arrangements must be made for her to be reviewed at the hospital, usually DAU (LW out of hours).

Women with pre-eclampsia on anti-hypertensive medication will be managed in hospital until day 4. On discharge she will have been given a postnatal BP management plan. When the community midwife checks her blood pressure she/he should reduce the woman's anti-hypertensive medication according to this plan until the BP is <130/80mmHg off treatment. Women on medication should have alternate day BP checks with the community midwife until off medication. If the woman is still on medication 12 days after delivery she must be told to arrange an appointment with her GP on day 13/14. Her GP should then manage her medication and will be responsible for prescribing any on-going anti-hypertensive medication.

If at any check the woman has raised blood pressure >149/99mmHg, or symptoms, arrangements must be made for her to be reviewed at the hospital usually DAU (LW out of hours).

If the woman is still on anti-hypertensive medication on day 12, the community midwife must ensure that the woman's current antihypertensive regime is clearly documented on her EPR record.

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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

9.0 References

- 9.1 NICE clinical guideline NG133 Hypertension in pregnancy: diagnosis and management (updated April 2023) <https://www.nice.org.uk/guidance/ng133>
- 9.2 MBRRACE-UK Saving Lives, Improving Mothers' Care April 2024 <https://www.npeu.ox.ac.uk/mbrrace-uk/data-brief/maternal-mortality-2020-2022#causes-of-maternal-deaths-uk-2020-2022> .
- 9.3 The Magpie Trial Collaborative Group (2002). Do women with pre-eclampsia and their babies benefit from magnesium sulphate? The Magpie Trial: a Randomized placebo-controlled trial. The Lancet 359 1877-1890
- 9.4 RCOG Green Top Guideline No: 7 Antenatal Corticosteroids to reduce neonatal Morbidity and Mortality (July 2022) <https://www.rcog.org.uk/guidance/browse-all-guidance/green-top-guidelines/antenatal-corticosteroids-to-reduce-neonatal-morbidity-and-mortality-green-top-guideline-no-74/>
- 9.5 Chronic Hypertension in Pregnancy and the Risk of Congenital Malformations: A Cohort Study; Bateman B, Huybrechts K, Fischer M, Seely E, Ecker J, Oberg A, Franklin J, Mogun H, Hernandez-Diaz S; American Journal of Obstetrics and Gynaecology (Sep 2014)

10.0 Monitoring Appendices and tables

Compliance with this guideline will be monitored using an audit tool. Results will be fed back at the Maternity & Children's Services Clinical Governance forum. Where monitoring has identified deficiencies an action plan will be developed and changes implemented as appropriate.

Author:	S Wong, S Williams	Date:	March 2026
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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Appendix 1: Indication for early delivery in a woman with pre-eclampsia who require in-patient management

Indication for early delivery in a woman with pre-eclampsia who require in-patient management

Name:

Date of birth:

Hospital no:

The above patient is an in-patient with pre-eclampsia and ideally should be delivered after 34+0 weeks. However, in some circumstances earlier delivery will be considered /advised.

The woman will be reviewed by a registrar or consultant each day. If the woman's clinical condition deteriorates between reviews the midwives will request extra medical review. If the clinical situation changes earlier delivery may be advised.

Acute circumstances, in which early delivery may be required include:

- Abnormal CTG/reverse end-diastolic flow in umbilical artery doppler
- Significant PV bleeding/placental abruption
- HELLP
- Refractory hypertension despite usual management
- Symptoms of deteriorating pre-eclampsia **or** signs of suggestive of imminent eclampsia such as clonus/de-saturating below 90%. These women will require urgent transfer to the Labour Ward and magnesium sulphate infusion before delivery.

If any of these acute events occur the woman must be urgently reviewed by a registrar, and management discussed with either the woman's own consultant, or the duty consultant. This discussion should be documented in the woman's EPR record.

Consultant name:.....

Consultants signature:.....

Date:.....

This document should be included on the woman's EPR record by the responsible consultant within 24 hours of admission.

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Appendix 2: Discharge to GP letter

Label

Definitions

Chronic hypertension present at booking <20/40

Gestational hypertension-new hypertension presenting after 20/40

PET new hypertension presenting >20/40 with significant proteinuria

Date

This patient is known to have:

PET / CHRONIC HYPERTENSION / GESTATIONAL HYPERTENSION (CIRCLE)

Date of admission _____ Date of discharge _____

Her antihypertensive medication on day of discharge was _____

Please tick appropriate

☐ **CHRONIC HYPERTENSION**

Check B/P on day 4

- A. If 140/90 and woman well no further checks required
- B. If B/P 141-150 over 91-100 CHECK B/P every 2 days
- C. B/P 150/100 call DAU whilst with the woman for advice
- D. Review by GP at two weeks postnatal

☐ **GESTATIONAL HYPERTENSION**

Check B/P on day 4

- A. Refer to GP if B/P falls below 140/90 mmHg to consider reducing antihypertensive medication.
- B. Check B/P on alternate days if antihypertensive treatment is changed after birth.
- C. Refer to DAU if woman unwell or B/P >149/99

☐ **POSTNATAL PRE ECLAMPSIA**

- A. Check B/P at home on first day after discharge
- B. If B/P is <150/100 no further action required. If symptomatic or B/P raised refer to DAU
- C. If B/P 130/80-149/99 advise woman to continue on current regime
- D. If B/P <130/80 to see GP for review of medication.
- E. If medication reduced check B/P 2 days after reduction.

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 1: Antenatal risk reduction

Educate women on early recognition of signs and symptoms of PET

All pregnant women will be advised to seek immediate advice if they experience symptoms of PET:

- Severe headache
- Problems with vision (blurring/flushes)
- Severe pain below the ribs
- Vomiting

Advise Aspirin 75mg OD from 12 weeks until 36 weeks if 1 or more high risk factor PET or 2 or more moderate risk factors, unless advised to take sooner.

Women at high risk of PET:

- Hypertensive disease in previous pregnancy
- Chronic renal disease
- Autoimmune disease e.g. SLE
- Type 1/ Type 2 diabetes
- Chronic hypertension

Women at moderate risk of PET:

- 1st Pregnancy
- ≥ 40 years
- Pregnancy interval > 10 years
- BMI ≥ 35kg/m²
- Family history of PET
- Multiple pregnancy

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 2: Classification of hypertensive disorders and summary of antenatal antihypertensive options

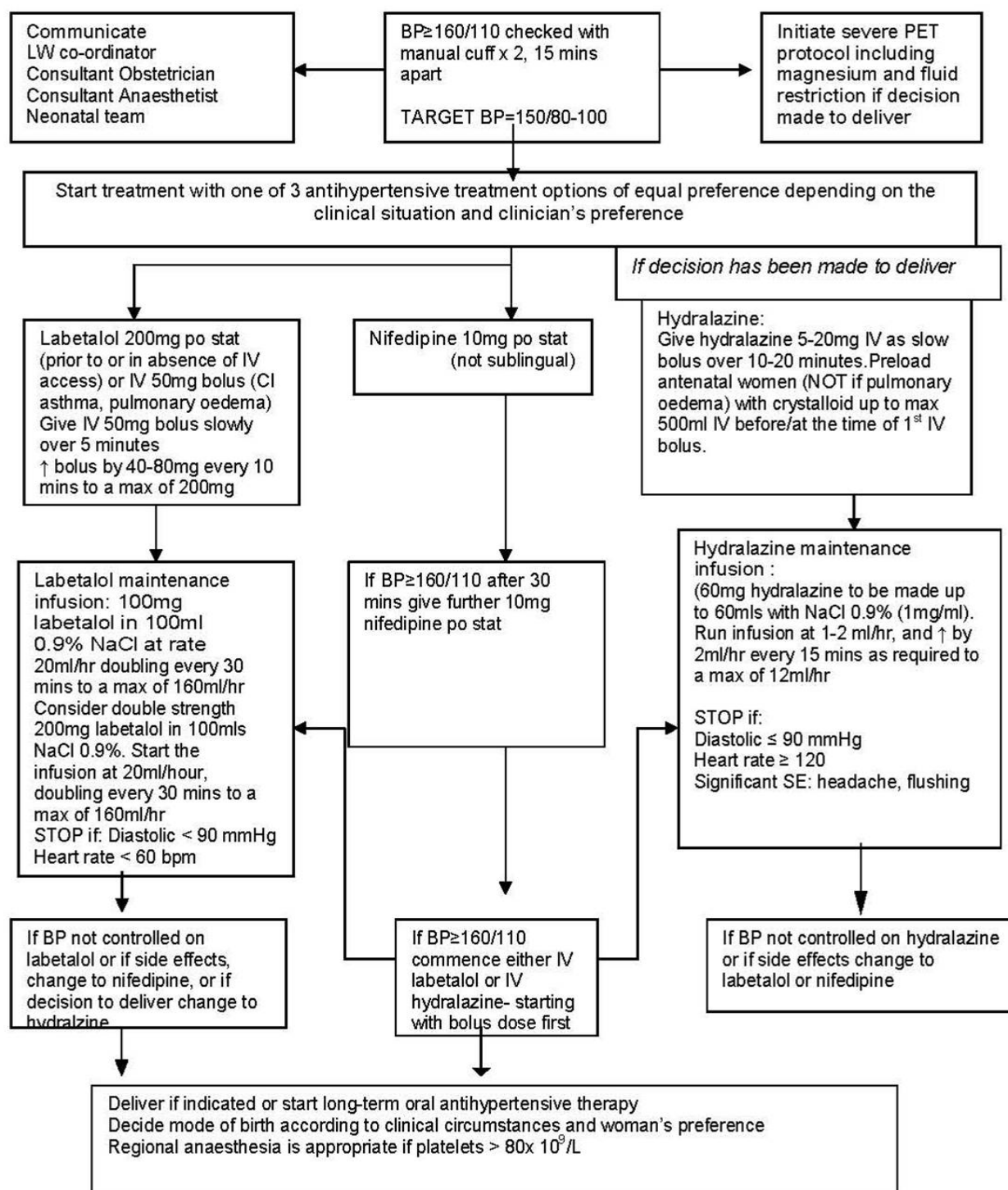
Diagnosis and classification	Choice 1	Choice 2	Choice 3
Chronic hypertension (hypertension at booking or ≤ 20 weeks or if already on antihypertensive therapy)	Labetalol (mixed alpha and beta blocker) dose:	Nifedipine (Adalat® Retard) dose:	Methyldopa (centrally acting) dose:
Gestational hypertension (new hypertension ≥ 20 weeks without significant proteinuria)	100mg BD increasing to a max 800mg a day in divided doses Labetalol is licensed for use in pregnancy	10mg BD to a max 80mg a day in divided doses Nifedipine is not licensed for use in pregnancy	250mg TDS increasing to max 3g a day in divided doses Methyldopa is licensed for use in pregnancy
Pre-eclampsia (new hypertension ≥ 20 weeks with significant proteinuria)			
Comment	<i>Contraindications:</i> Asthma, bradycardia, pulmonary oedema <i>Side effects:</i> Maternal bradycardia, tiredness Caution: DM	<i>Contraindications:</i> Advanced aortic stenosis, <i>Side effects:</i> Headache, flushing	<i>Contraindications:</i> Liver disease, depression, acute porphyria <i>Side effects:</i> Drowsiness, depression

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

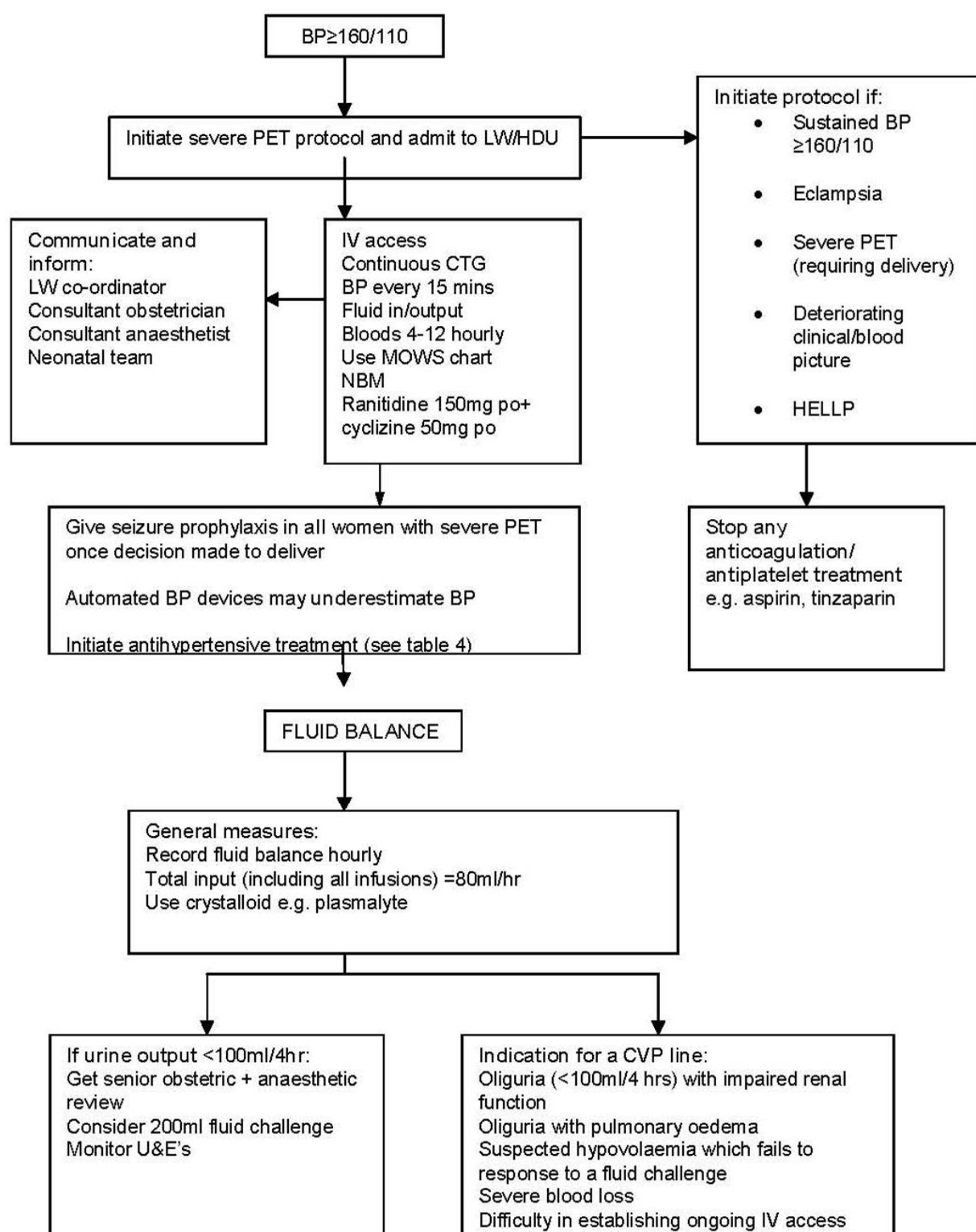
Table 3: Management of antenatal hypertension

ACTION	CHRONIC HYPERTENSION	GESTATIONAL HYPERTENSION	PRE- ECLAMPSIA
Admit to hospital	For severe hypertension ($\geq 160/110$)	For severe hypertension ($\geq 160/110$)	Admit moderate and severe cases
Treat	- If on pre-conception AHT ensure use of a drug that reduces fetal risks and maternal side effect profiles - Aim for a BP of $<140/90$mmHg - Consider antenatal referral to a HT specialist/obstetric medicine clinic	Use AHT to keep: BP 140/90mmHg	Use AHT to keep: BP $<140/90$mmHg
BP measurement Mild: 140-149/ 90-99 Moderate: 150-159/100-109 Severe: $>160/110$	- Severe – BP check 4 x a day - If BP controlled at booking ($<150/100$) measure BP 2-4 weekly. If it remains controlled then increase frequency depending on clinical picture	- Mild x 1 week - Moderate x 2 week - Severe $> x 4$ a day	- Mild x 3/week - Moderate 4hrs - Severe 4hrs
Test for proteinuria	At each antenatal visit using urine dip stick or urine PCR.	At each antenatal visit using urine dip stick or urine PCR.	Once significant proteinuria found, no need to repeat quantification
Blood tests (FBC, U&E, Creatinine, LFT's and Clotting if platelets $< 100 \times 10^9/L$)	- At Booking - No need to repeat if normal unless signs/symptoms of superimposed PET	- Test at presentation for mod/severe HT - Re-test depending on clinical picture	PET bloods 2-3 x week depending on clinical circumstances

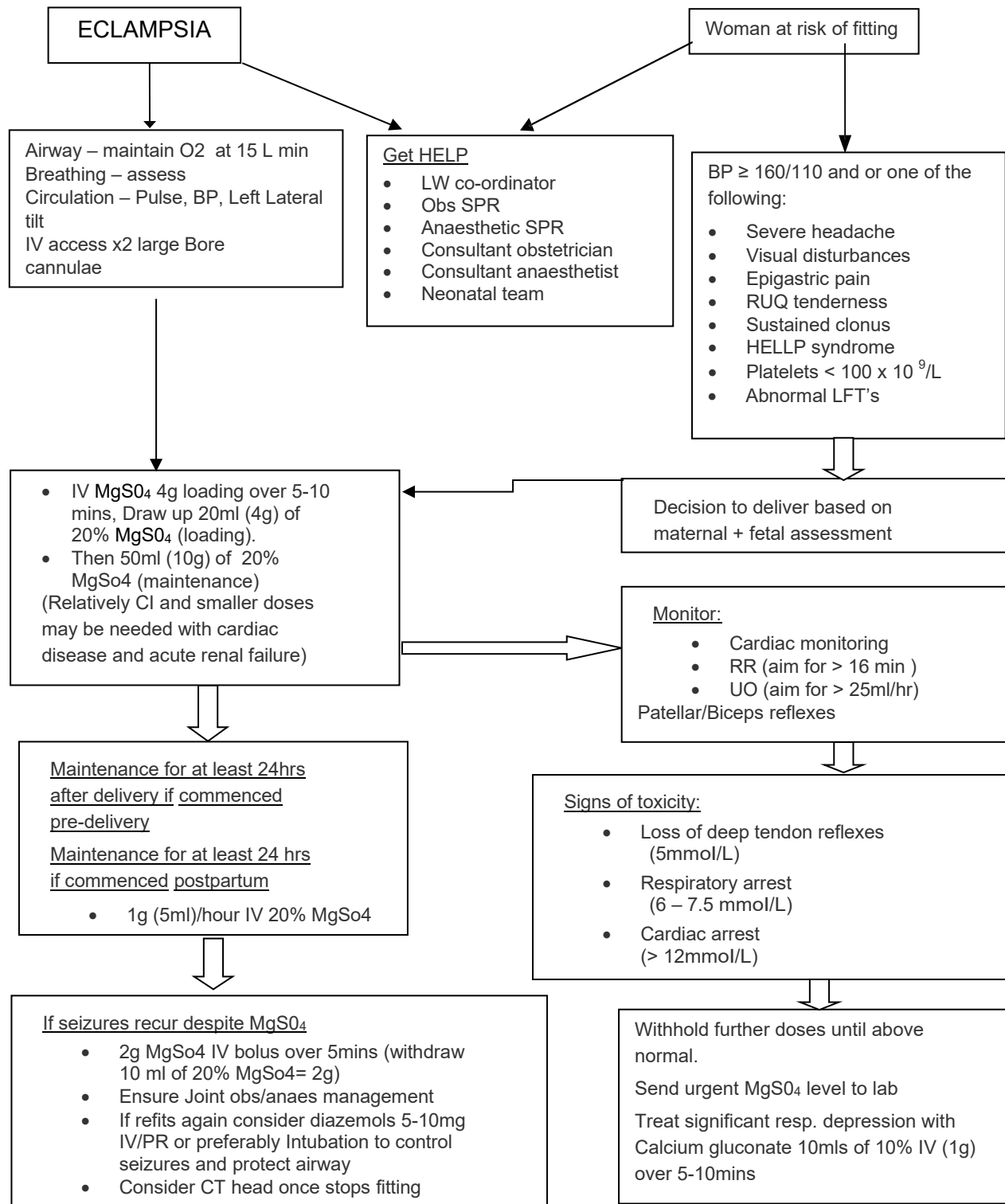
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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 4: Diagnosis and management of severe hypertension: Antihypertensive treatment options

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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 5: Management of severe hypertension: assessment, diagnosis and fluid balance

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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 6: Management of severe hypertension: Eclampsia:**Seizure prophylaxis and treatment**

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Policy Lead:	Group Director Urgent Care	Version:	V6.3
Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 7: Fetal assessment and delivery planning

	Fetal Assessment	Delivery Planning
Chronic Hypertension	- Consider uterine artery Doppler screening at 23-24/40 USS growth, AFI, umbilical artery Doppler:- 28-30weeks & 32-34 weeks CTG monitoring:- - Only if reduced Fetal movement	- If BP < 160/110 with or without AHT < 37/40 – No indication for delivery >37/40 – Timing based on individual case and following discussion with senior obstetrician and neonatologist
Gestational Hypertension	If diagnosed < 34/40 :- - USS growth, AFI, Doppler and if normal do not routinely repeat If diagnosed > 34/40 :- - Routine USS not indicated CTG monitoring:- - Only if reduced Fetal movement	If BP ≤ 160/100 with/without AHT <37/40 – No indication for delivery > 37/40 timing based on individual case and following discussion with senior obstetrician and neonatologist
Pre- eclampsia	USS growth, AFI and umbilical artery Doppler - At presentation and repeat 2-4 weekly depending on clinical picture CTG monitoring:- - At diagnosis and repeat daily if inpatient and weekly if outpatient - Consider steroids for all diagnosis of PET < 34 weeks - Involve neonatologist in joint discussions regarding timing of preterm deliveries	< 34/40 – Manage conservatively where possible Unless - Severe HT refractory to treatment - Maternal/fetal indication for delivery as specified in the consultant plan > 34/40 – If severe deliver after steroids 34+⁰ – 36+⁶ weeks – - offer delivery to women with complicated moderate HT depending on maternal/fetal condition, risk factors and cot availability ≥37/40 – uncomplicated mild/moderate HT following discussion with senior obstetrician

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 8: Summary of postnatal hypertension management

Diagnosis	BP measurement	Treatment	Discharge
Chronic Hypertension	4 hourly 1st day Daily 2nd day At least once during days 3-5 as clinically indicated	Aim to keep BP = 140/90 Continue AN AHT treatment (change if on methyldopa within 2 days of birth and re-start pre-pregnancy AHT if safe for breastfeeding)	Review long- term AHT at 2 weeks Offer follow- up with pre- pregnancy medical team at 6-8 weeks for long-term BP follow-up
Gestational Hypertension	4 hourly 1st day Daily 2nd day At least once during days 3-5 as clinically indicated	Continue AN AHT treatment (change if on methyldopa within 2 days of birth) If BP $\leq$ 140/90 consider reducing dose Reduce dose if BP $\leq$ 130/80 If previously untreated and BP $\geq$ 149/99 consider starting AHT	If on AHT offer medical review at 2 weeks If on AHT at 2 weeks offer medical review at 6-8 weeks If on AHT at 6-8 weeks offer specialist referral
Pre- eclampsia	If no AN treatment measure BP: 4 x day whilst inpatient At least 1 x day during days 3-5 Alternate days until normal Start AHT Rx if BP $\geq$ 150/100 If had AN treatment measure BP: 4 x day whilst inpatient Every 1-2 days for up to 2 weeks until off Rx and BP normal	If on AN AHT: Continue AHT (change if on methyldopa within 2 days of birth) Consider reducing AHT if BP $\leq$ 140/90 Reduce AHT if BP $\leq$ 130/80	Discharge only if: No symptoms PET BP with/without AHT $\leq$ 150/100 Blood tests are normal or improving If on AHT @ 2 weeks offer medical review Offer all women a medical review @ 6-8 weeks If still on AHT @ 6-8 weeks, offer specialist referral

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 9: Antihypertensive therapy and breastfeeding

No known adverse effects - Assess wellbeing of baby daily for at least 2 days	Dose	Comments
Labetalol	100mg BD, increase to a max of 800mg a day in divided doses	
Nifedipine Adalat Retard ®	10mg BD, increase to max 40mg BD	
Enalapril	5mg OD, increase to 20mg OD if required	Check maternal U+Es one week after starting dose
Captopril	12.5mg BD, increase to 25mg BD	Check maternal U+Es one week after starting dose
Atenolol	25-50mg OD, increase to max 100mg a day in divided doses	
Metoprolol	100mg OD, increase to max 400mg a day in divided doses	
Notes: For all babies, whose mothers are taking AHT in the postnatal period assess wellbeing of the baby especially adequacy of breastfeeding at least daily for the first 2 days after birth Insufficient evidence on the safety of ARB (Angiotensin receptor blockers) Amlodipine ACE other than Enalapril/ Captopril		

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Location:	Policy hub/ Clinical/ Maternity/ LIFESAVING / GL861		

Table 10: Recurrence risks of hypertension and long-term health risks

Future Risk	Hypertensive Disorder		
	Gestational Hypertension	Pre-eclampsia	Severe Pre-eclampsia, HELLP syndrome or eclampsia
Gestational hypertension in a future pregnancy	Risk ranges from about 1 in 6 (16%) to about 1 in 2 (47%)	Risk ranges from about 1 in 8 (13%) to about 1 in 2 (53%)	
Pre- eclampsia in future pregnancy	Risk ranges from 1 in 50 (2%) to about 1 in 14(7%)	Risk up to about 1 in 6 (16%) No additional risk if interval before next pregnancy < 10 years	If birth was needed before 34 weeks' risk is about 1 in 4 (25%). If birth was needed before 28 weeks is about 1 in 2 (55%).
Cardiovascular disease	Increased risk of hypertension and its complications.	Increased risk of hypertension and its complications.	Increased risk of hypertension and its complications.
End-stage kidney disease		If no proteinuria and no hypertension at 6-8 week postnatal, relative risk increased but absolute risk low. No follow up needed	
Thrombophilia		Routine screening not needed.	

Advise women who have had hypertensive disorder in pregnancy

- 1 in 5 chance of recurrence.
- Discuss ways to reduce risk of cardiovascular disorder e.g. quite smoking, healthy lifestyle and healthy weight (BMI 18.5-24.9kg/m²).

In women who needed early birth before 34 weeks due to pre-eclampsia or hypertension, consider pre-pregnancy counselling to discuss possible risks of recurrent hypertensive disorders.

Advise women with pre-eclampsia the likelihood of recurrence increases with inter-pregnancy interval of greater than 10 years.

Inform women with pre-eclampsia who have no proteinuria and no hypertension at the 6-8 weeks' postnatal review that although the relative risk of end-stage kidney disease is increased, the absolute risk remains low. No follow up is required.

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